

HANDOFFS / PATIENT EDUCATION / TEAMWORK / SAFETY

Nursing & Allied Health English

Speak clearly. Check understanding.
Confirm the next action.

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Eight scenarios. Eight extended conversations.

INSIDE THE BOOK

192 vocabulary entries / 128 reusable phrases
160 dialogue turns / 80 numbered dialogue gaps
Word banks and explained answer keys

Upper-intermediate to advanced (B2-C1). For nurses, therapists, and allied-health professionals communicating with patients, families, and clinical teams.

2026 edition

Eight scenarios. Eight skills.

Eight clinical communication simulations, from shift handoffs to patient explanations and safety reviews.

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Clear language is part of the care.

Clinical conversations move between technical detail and everyday language. A colleague needs a precise observation and an actionable request; a patient needs a usable explanation without unexplained shorthand. These fictional cases practice that change of register while keeping uncertainty, professional boundaries, and the patient's own account visible.

Lead with the important change

Identify yourself, the person concerned, the change, and the response you need. A well-organized report supports prompt attention; it must never delay urgent action while you perfect the wording.

"This is a change from the earlier assessment. I need a review now."

Name the source of information

Separate what you observed, what the patient reported, and what another record states. Preserve a discrepancy for appropriate review instead of silently choosing the version that looks most convenient.

"The record says none known; the patient reports a previous reaction."

Check the explanation, not the patient

Use ordinary language and small steps. A patient's own account of the agreed plan can reveal a misunderstanding that a polite nod leaves hidden. Clarify and check again without blame.

"I may not have explained that clearly. Let us go through that step again."

Close the loop with the receiving person

State who owns the next action and confirm that the other person has understood it. Sending a message, accepting a task, and completing that task are different events.

"You will follow the pending result and contact the responsible clinician. Is that correct?"

PROFESSIONAL BOUNDARIES

Original fictional language practice, not clinical training, medical advice, or a treatment protocol. No real patient information is used. Follow current local procedures, professional scope, applicable privacy rules, and qualified clinical judgment. UK sources are identified as such; their legal duties are not presented as universal rules.

The shift ends; the result is still pending

A Read the situation

Transfer an unfinished clinical follow-up with its current status, named owner, and confirmed next action.

In a simulation, outgoing nurse Maya hands over to incoming nurse Jonas at 7 p.m. A requested laboratory result is still pending; Maya has not seen a value. The responsible clinician has asked to be contacted when it becomes available. Jonas will take over checking the result and contacting that clinician through the agreed route. The team must also continue the existing care plan. The handoff does not establish a new diagnosis or replace local escalation procedures.

PEOPLE / Maya: Outgoing nurse | Jonas: Incoming nurse

TRANSFER THE TASK, NOT JUST THE INFORMATION

An experienced colleague may sound brief without intending to be dismissive. Ask for the missing owner or action directly. A concise handoff is useful only when the receiving person knows which facts are current and which responsibilities they are accepting.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What is known about the requested result?

- A** It has been reviewed by the responsible clinician.
- B** It was cancelled at the shift change.
- C** It is pending, with no value available to Maya.
- D** It is within the expected range.

2. Which responsibility is transferring?

- A** Choosing treatment from the expected result
- B** Replacing the existing care plan
- C** Declaring the patient ready for discharge
- D** Checking for the result and contacting the responsible clinician when available

3. What completes the verbal transfer?

- A** The shift clock reaches seven.
- B** The result remains in the electronic record.
- C** Jonas confirms the task and next action, and Maya checks that confirmation.
- D** Maya names the test without discussing follow-up.

[Answers and explanations: page 76.](#)

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

handoff

Transfer of relevant information and responsibility between care professionals.

IN USE give a structured handoff

shift overlap

A period when outgoing and incoming staff are both present.

IN USE use the shift overlap

patient identifier

An approved detail used to establish the correct person's identity.

IN USE verify patient identifiers

current status

The person's or task's condition at the stated present time.

IN USE summarize current status

baseline

A reference condition used when describing a change.

IN USE compare with the baseline

relevant history

Background information that bears on the present care situation.

IN USE provide relevant history

pending result

A requested result not yet available or finalized.

IN USE follow a pending result

outstanding task

An action that remains to be completed.

IN USE identify outstanding tasks

follow-up owner

The named person accountable for the next agreed action.

IN USE name the follow-up owner

receiving clinician

The professional taking over information or care responsibility.

IN USE confirm the receiving clinician

contingency

An agreed response if a specified situation occurs.

IN USE clarify the contingency

check-back

Repetition and confirmation used to verify a received message.

IN USE use a check-back

Precision check

Pending is not normal, negative, reviewed, or unimportant. State what has actually been received and who will follow it. A handoff framework organizes communication; it does not supply missing observations or authorize a clinical decision.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

SBAR

Situation, background, assessment, and recommendation or request.

IN USE structure the report with SBAR

I-PASS

A structured handoff framework expanded in the precision note below.

IN USE use an I-PASS handoff

care plan

The documented, agreed approach to a person's care.

IN USE continue the care plan

action list

A stated set of tasks with relevant responsibility and timing.

IN USE review the action list

clinical priority

The relative urgency established through appropriate clinical assessment.

IN USE communicate clinical priority

escalation route

The designated path for obtaining further clinical attention.

IN USE confirm the escalation route

read-back

Repeating supplied information to check its accuracy.

IN USE complete a read-back

acknowledgment

Confirmation that a message has been received.

IN USE obtain an acknowledgment

responsibility transfer

Explicit movement of a defined duty to another professional.

IN USE document responsibility transfer

continuity of care

Coordination that connects care across people, shifts, or settings.

IN USE support continuity of care

source record

The original record from which information is taken.

IN USE check the source record

time stamp

A recorded time attached to an observation or action.

IN USE include the time stamp

Precision check

I-PASS: illness severity; patient summary; action list; situation awareness and contingency plans; synthesis by receiver. Use the locally adopted format. Acknowledgment confirms receipt; a check-back also verifies meaning. Urgent changes still require the appropriate immediate response.

Phrases you can actually use

C Notice the language / 1 of 2

OPEN THE TRANSFER

"I am handing over one result that is still pending."

TIME THE REPORT

"This is the status at the seven o'clock handoff."

NAME THE REQUEST

"The responsible clinician asked to be contacted when it is available."

ACCEPT THE TASK

"I will check for the result and contact the responsible clinician."

CONFIRM IDENTITY

"Let us verify the patient identifiers before discussing the record."

SEPARATE KNOWN FROM UNKNOWN

"I have not seen a result value yet."

GIVE THE OWNER

"You are taking over that follow-up from this handoff."

CHECK UNDERSTANDING

"Please repeat the outstanding action so we can confirm it."

Pending versus reviewed

These describe different stages; neither should be substituted for the other.

I will versus someone will

A named commitment is easier to act on than an unspecified expectation.

Stable

Use only when supported by an actual clinical assessment, not as a filler word.

Phrases you can actually use

C Notice the language / 2 of 2

CORRECT A MISMATCH

"Not reviewed; it is still pending."

USE THE SOURCE

"Let us check the current record rather than rely on an old summary."

AVOID A GUESS

"We cannot describe a value that has not been received."

DOCUMENT THE TRANSFER

"Record the task, receiving professional, and agreed next action."

PRESERVE THE PLAN

"The existing care plan remains in place unless appropriately changed."

KEEP URGENCY VISIBLE

"A change in condition follows the clinical escalation process."

NAME THE CONTINGENCY

"Which agreed route applies if the expected follow-up cannot be completed?"

CLOSE THE LOOP

"That is correct; you have the pending-result follow-up."

Handoff or handover

Both terms are common; local usage varies.

Read-back versus completion

Repeating an action does not mean the action has already occurred.

At seven

A time reference limits the status claim to the information then available.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which opening preserves the known facts?

- A** The shift has changed, so the original request no longer applies.
- B** At seven, the result is still pending; Jonas will follow it and contact the responsible clinician.
- C** At seven, the result was reviewed and no action was needed.
- D** The laboratory will probably call, so no owner is necessary.

2. Jonas says, "I will review the normal result." What needs correction?

- A** The clinician has already changed treatment.
- B** No value is available, so normal is unsupported.
- C** The result is definitely abnormal.
- D** The follow-up task has been cancelled.

3. Which response is a useful check-back?

- A** I will wait for the next handoff to discuss ownership.
- B** I will check for the result and contact the responsible clinician when it is available.
- C** I heard a test mentioned.
- D** The computer probably knows what to do.

4. What should happen if an urgent change occurs before the result?

- A** Use the appropriate clinical escalation process without waiting for that result.
- B** Wait because result follow-up was assigned first.
- C** Call the result normal to simplify the handoff.
- D** Treat the communication framework as a replacement for local procedures.

[Answers and explanations: page 76.](#)

The shift ends; the result is still pending

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

pending result / continuity of care / follow-up owner / check-back / care plan / outstanding task / current status / escalation route / source record / responsibility transfer

- 01 **Maya:** Before I leave, we need to hand over one unfinished follow-up. We have checked the patient identifiers, and this report reflects the position at seven.
- 02 **Jonas:** Go ahead with the ¹ _____. I have the correct record open, but I do not want to mistake an earlier note for the latest information.
- 03 **Maya:** The requested laboratory result has not arrived. I have not seen a value, and the responsible clinician asked to be contacted when it becomes available.
- 04 **Jonas:** So it remains a ² _____, not a reviewed result. I will not describe it as normal just because there is no new value on this screen.
- 05 **Maya:** Correct. The request to contact the clinician still stands. The shift change should not leave the task sitting between two people who each assume the other will act.
- 06 **Jonas:** I will be the ³ _____ from this handoff. I will check for the result and contact the responsible clinician through our agreed route when it is available.
- 07 **Maya:** That is the action I am transferring. The existing care plan continues; this conversation does not introduce a different treatment or a new assessment.
- 08 **Jonas:** I will retain the ⁴ _____ and follow any properly communicated changes. I also want to keep this result task separate from other observations and responsibilities on the shift.
- 09 **Maya:** The current record contains the request and the relevant background. If a copied summary conflicts with it, we need to clarify that difference rather than choose the shorter version.
- 10 **Jonas:** I will use the ⁵ _____ to verify the request. A familiar phrase in yesterday's summary is not enough to establish what is required now.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 10.

- 11 **Maya:** Exactly. If the patient's condition changes before the result arrives, that change needs its own assessment and response. We cannot wait merely because this test is outstanding.
- 12 **Jonas:** The ⁶ _____ remains available for that concern. The pending-result task is not a reason to postpone attention to a new clinical change.
- 13 **Maya:** Please repeat the follow-up in one sentence. I want to check that I have given you a clear action rather than just another piece of information.
- 14 **Jonas:** Here is my ⁷ _____: I will check for the outstanding result and contact the responsible clinician when it becomes available, using the agreed route.
- 15 **Maya:** That matches my request. If the follow-up cannot be completed through that route, use the applicable local process rather than leave an unacknowledged message indefinitely.
- 16 **Jonas:** I will keep the ⁸ _____ visible until the required follow-up is complete. An acknowledgment that I heard you is not evidence that the clinician has reviewed the result.
- 17 **Maya:** Good. We should record who accepted the task and when. That will help the next person distinguish this transfer from an assumption made after I left.
- 18 **Jonas:** I will document the ⁹ _____ with the relevant time. The record will state the actual arrangement, not imply that the result was already available.
- 19 **Maya:** We agree, then: no value yet, your follow-up from seven, and contact with the responsible clinician when the result arrives. The other care responsibilities remain in place.
- 20 **Jonas:** Agreed. That supports ¹⁰ _____ across the shift change. I have the request, the source, and the next action, rather than simply a test name.

ANSWERS / EXPLANATIONS: PAGE 77

1. current status | 2. pending result | 3. follow-up owner | 4. care plan | 5. source record | 6. escalation route | 7. check-back | 8. outstanding task | 9. responsibility transfer | 10. continuity of care

Seen by whom?

F Say it

A scan report is available, but the responsible clinician has not yet reviewed it. The incoming therapist agrees to contact that clinician. No clinical interpretation is supplied.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

action / contact / interpretation / pending

Therapist: "The report is available, but clinical review is ¹ _____."

Nurse: "Who owns the next ² _____?"

Therapist: "I will ³ _____ the responsible clinician."

Nurse: "Do not add an unsupported ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 77

1. pending | 2. action | 3. contact | 4. interpretation

Rehearse with a fixed script

1. Check your answers, then read turns 1-10 with a partner using the corrected words.
2. Switch roles for turns 11-20. Stress words that distinguish facts from conditions or estimates.
3. Complete and check the four-line exchange. Read it twice, switching roles and keeping the printed facts.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

A change needs a clear request

A Read the situation

Report a new concern with its time, source, and comparison, then confirm an appropriate clinical response.

In a simulation at 10:20 a.m., patient Alex tells nurse Priya, "I feel worse than earlier." Priya observes that Alex is now stopping mid-sentence to catch a breath, unlike at 10 a.m. The cause is not established. Priya uses the local urgent-response process immediately and communicates with responding clinician Dan. The exercise practices that exchange during the response, not a reason to delay it. No treatment, diagnosis, or numerical threshold is supplied.

PEOPLE / Priya: Nurse | Dan: Responding clinician

DIRECTNESS CAN BE A SAFETY SKILL

A clear request for urgent review is not an accusation. State the change and what you need without softening the message into a casual suggestion. Professional politeness should not obscure urgency, and a senior colleague should acknowledge the concern explicitly.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What changed between the supplied observations?

- A** A new numerical result became available.
- B** The patient withdrew the concern.
- C** Alex now stops mid-sentence to catch a breath.
- D** A diagnosis was confirmed.

2. What is the source of "I feel worse than earlier"?

- A** A laboratory result
- B** Dan's completed assessment
- C** A copied discharge note
- D** Alex's own report

3. What is Priya already doing?

- A** Deferring the concern until the next shift
- B** Using the local urgent-response process
- C** Waiting to complete a perfect script
- D** Assigning a diagnosis from the dialogue

Answers and explanations: page 78.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

patient-reported symptom

A change or experience described by the patient.

IN USE record a patient-reported symptom

clinical observation

A finding noticed or measured during clinical care.

IN USE communicate a clinical observation

change from baseline

A difference from the person's relevant reference condition.

IN USE report a change from baseline

onset

The beginning of a symptom or change.

IN USE clarify symptom onset

time course

The pattern of a condition or change over time.

IN USE describe the time course

deterioration

Worsening of a person's clinical condition.

IN USE recognize possible deterioration

assessment finding

Information obtained through an appropriate clinical assessment.

IN USE report assessment findings

clinical concern

A reason for professional attention to possible patient risk.

IN USE state the clinical concern

urgent review

Prompt assessment requested because of a clinical concern.

IN USE request an urgent review

response activation

Initiating the applicable process for clinical assistance.

IN USE confirm response activation

clinical uncertainty

A stated limit on what is known about a clinical situation.

IN USE communicate clinical uncertainty

differential diagnosis

Possible explanations considered through qualified clinical assessment.

IN USE discuss a differential diagnosis

Precision check

Describe the actual change and its time before adding an interpretation. A patient-reported symptom and your own observation can both matter, but they are different sources. Do not turn a concern into a confirmed diagnosis merely to sound confident.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

situational awareness

Shared understanding of the current conditions and relevant risks.

IN USE maintain situational awareness

call-out

A clear announcement of important information to the team.

IN USE make a focused call-out

closed-loop communication

An exchange in which receipt and meaning are confirmed.

IN USE use closed-loop communication

assertive statement

A direct, respectful expression of a concern or need.

IN USE make an assertive statement

safety concern

A condition that may expose someone to harm.

IN USE raise a safety concern

chain of command

The designated sequence for escalating unresolved concerns.

IN USE use the chain of command

reassessment

A further assessment after time, intervention, or a change.

IN USE communicate reassessment findings

response time

The interval between a request and the relevant response.

IN USE record the response time

bedside review

Clinical assessment with the patient present.

IN USE arrange a bedside review

monitoring plan

The agreed approach to observing and responding to clinical status.

IN USE clarify the monitoring plan

clinical rationale

The professional reasoning supporting a clinical decision.

IN USE explain the clinical rationale

escalation record

Documentation of the concern, contacts, response, and relevant times.

IN USE complete the escalation record

Precision check

Urgent language should accompany the appropriate response, not replace it. Follow local procedures and professional judgment. This fictional exchange supplies no treatment, monitoring interval, diagnostic conclusion, or universal trigger for activating a clinical team.

Phrases you can actually use

C Notice the language / 1 of 2

STATE THE CHANGE

"This is different from the observation at ten o'clock."

DESCRIBE WHAT YOU SEE

"Alex is now stopping mid-sentence to catch a breath."

CONFIRM ACTION

"I have activated the local response process."

GIVE THE TIME

"I observed this change at 10:20."

ATTRIBUTE THE REPORT

"Alex says, 'I feel worse than earlier.'"

REQUEST REVIEW

"I need an urgent review now."

AVOID AN UNSUPPORTED DIAGNOSIS

"The cause has not been established."

KEEP THE CONCERN CLEAR

"I am concerned about the change, not just the wording in the earlier note."

Worse

Attribute the patient's word and add relevant observations rather than replacing it with a diagnosis.

Now versus earlier

These time markers make the comparison actionable.

Could you perhaps

Excessive softening can hide an urgent request.

Phrases you can actually use

C Notice the language / 2 of 2

ASK FOR ACKNOWLEDGMENT

"Please confirm that you have received the urgent request."

SUMMARIZE THE COMPARISON

"Earlier speech was uninterrupted; now Alex pauses to catch a breath."

ESCALATE AN UNRESOLVED CONCERN

"The concern remains unresolved; I am using the next appropriate route."

RECORD THE RESPONSE

"Document the concern, contact time, and actual response."

CORRECT MINIMIZATION

"The earlier assessment does not describe what I am seeing now."

CLARIFY RESPONSIBILITY

"Who is taking the clinical review, and what response is underway?"

STAY WITHIN EVIDENCE

"I can report the observation without claiming a cause."

MAINTAIN CONTINUITY

"Any further change needs to reach the responding team promptly."

No diagnosis yet

Uncertainty about cause does not mean there is no need for attention.

Response activated

This is an action status, not proof that an assessment has finished.

Escalation

Escalating an unresolved concern is not the same as blaming a colleague.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which opening is most useful during the response?

- A** The cause is confirmed because the patient said worse.
- B** At 10:20 Alex is stopping mid-sentence to catch a breath, unlike at ten; I need urgent review.
- C** Alex was assessed earlier, so this is probably unchanged.
- D** I have a possible administrative issue whenever you are free.

2. Which statement properly separates sources?

- A** The laboratory reports that Alex feels worse.
- B** Dan observed the change at ten before it occurred.
- C** The patient's words are my confirmed diagnosis.
- D** Alex reports feeling worse; I observe pauses to catch a breath.

3. Dan has not acknowledged the request. Which response fits the brief?

- A** Continue the appropriate urgent escalation process rather than assume silence means acceptance.
- B** Treat the request as accepted because it was sent.
- C** Wait until the next shift to check receipt.
- D** Replace the concern with a less urgent message.

4. What should the later record describe?

- A** An earlier status copied as though current
- B** An assessment marked complete before it happened
- C** Actual observations, attributed reports, relevant times, and the response taken
- D** A diagnosis added to make the narrative decisive

Answers and explanations: page 78.

A change needs a clear request

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

time course / closed-loop communication / escalation record / patient-reported symptom / urgent review / clinical concern / change from baseline / situational awareness / reassessment / clinical uncertainty

- 01 **Priya:** Dan, I have activated our urgent-response process for Alex. At ten twenty, Alex is stopping mid-sentence to catch a breath. This differs from the observation at ten.
- 02 **Dan:** I have received the request for ¹ _____. I am responding now. Tell me what Alex reported and what you observed, without waiting to establish a cause.
- 03 **Priya:** Alex said, "I feel worse than earlier." I observed the pauses while Alex was speaking to me. I have not established what is causing the change.
- 04 **Dan:** That separates the ² _____ from your own finding. Keep the patient's words attributed rather than rewriting them as a diagnosis that has already been confirmed.
- 05 **Priya:** At ten, Alex spoke without those pauses during my conversation. I am concerned that the earlier note could make this sound like an unchanged situation.
- 06 **Dan:** Your comparison identifies a ³ _____. The earlier entry provides context, but it cannot substitute for assessment of what is happening now.
- 07 **Priya:** I need the team to understand the urgency. I do not want my request to sound like a routine update that can wait until the next round.
- 08 **Dan:** Your ⁴ _____ is clear. Keep the request for review now at the front of the message; the receiving team should not have to infer urgency from the older note.
- 09 **Priya:** Thank you. I will communicate any further change through the response already underway. I will not delay that process while trying to produce a perfectly worded summary.
- 10 **Dan:** Maintain ⁵ _____ across the team. Everyone involved needs the present concern and response status, not just the reassuring language of an older entry.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 19.

- 11 **Priya:** If a request is not acknowledged, I should not assume that pressing send means the receiving person has accepted responsibility for responding.
- 12 **Dan:** Correct. Use ⁶ _____ and the appropriate escalation process. The route must remain responsive to the concern; an unanswered message is not a completed clinical review.
- 13 **Priya:** For the handover to the responding team, I can give the patient identifiers, the two observation times, the patient's words, and the action already taken.
- 14 **Dan:** Include the ⁷ _____ clearly. In this case, the earlier observation and the change at ten twenty are more useful than an unspecific statement that things changed sometime.
- 15 **Priya:** I also want to avoid writing a possible cause as if it were established. We have a concern requiring assessment, not a diagnosis from this conversation.
- 16 **Dan:** State that ⁸ _____ explicitly. Not knowing the cause does not reduce the importance of reporting the change or obtaining the appropriate assessment.
- 17 **Priya:** I will record the observation and the response activation with their times. Later actions should be added as they actually occur, not as though they already happened.
- 18 **Dan:** The ⁹ _____ should reflect that sequence. Documenting a request for assessment and documenting the completed assessment are different entries about different events.
- 19 **Priya:** We have the urgent request acknowledged and the response underway. The next team should receive the same factual comparison without an invented result or treatment instruction.
- 20 **Dan:** Agreed. Any ¹⁰ _____ findings will be communicated through the clinical process. Keep the current concern visible as the responding team takes over; the earlier note must not obscure the change.

ANSWERS / EXPLANATIONS: PAGE 79

1. urgent review | 2. patient-reported symptom | 3. change from baseline | 4. clinical concern | 5. situational awareness | 6. closed-loop communication | 7. time course | 8. clinical uncertainty | 9. escalation record | 10. reassessment

An earlier note is not a current finding

F Say it

At 2 p.m., a therapist identifies a new change during a simulation and uses the appropriate response route. A colleague refers to an unchanged status recorded at noon.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

assessment / change / underway / earlier

Therapist: "The noon entry is an ¹ _____ observation."

Colleague: "The two o'clock finding describes a ² _____."

Therapist: "The appropriate response is already ³ _____."

Colleague: "We must not substitute the old note for a current ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 79

1. earlier | 2. change | 3. underway | 4. assessment

Rehearse with a fixed script

1. Check your answers, then read turns 1-10 with a partner using the corrected words.
2. Switch roles for turns 11-20. Stress words that distinguish facts from conditions or estimates.
3. Complete and check the four-line exchange. Read it twice, switching roles and keeping the printed facts.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

The allergy field and the patient disagree

A Read the situation

Communicate conflicting allergy information without deleting a report, diagnosing a reaction, or guessing a medication decision.

In a simulation, the medication record says "no known drug allergies." Patient Rosa reports a rash after an antibiotic several years ago, but cannot recall the name or exact date. No current reaction is reported. Nurse Mei contacts pharmacist Sam through the local medication-safety process before the unresolved discrepancy is treated as settled. The available facts do not establish the cause of the historical rash or authorize a drug choice, dose, or administration decision.

PEOPLE / Mei: Nurse | Sam: Pharmacist

MAKE UNCERTAINTY SPECIFIC

A patient may use allergy for several kinds of past experience. Listen without dismissing the account or confirming a diagnosis beyond the evidence. Explain what needs clarification and preserve the source of each detail so the next professional can assess it appropriately.

Briefing checks

Circle one answer in each item. Use the briefing.

1. Which conflict needs review?

- A** The record supplies a verified date that Rosa disputes.
- B** The record says no known drug allergies, while Rosa reports a previous rash after an antibiotic.
- C** Two clinicians have confirmed opposite diagnoses.
- D** Rosa reports a current reaction to a named drug.

2. Which details remain unknown?

- A** Whether Rosa described a rash
- B** Whether the record contains an allergy entry
- C** Whether Mei contacted the medication-safety process
- D** The antibiotic name and exact date

3. What can this language exercise establish?

- A** That the historical rash was definitely allergic
- B** That the patient has no relevant history
- C** How to communicate the discrepancy accurately for appropriate review
- D** Which drug should be administered

Answers and explanations: [page 80](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

allergy status

The documented position on a person's known or suspected allergies.

IN USE verify allergy status

drug allergy

A harmful immune-mediated response to a medicine.

IN USE assess a suspected drug allergy

adverse drug reaction

A harmful, unintended response to a medicine.

IN USE document an adverse drug reaction

intolerance

An unwanted response that is not necessarily an immune-mediated allergy.

IN USE clarify a reported intolerance

hypersensitivity

A reproducible adverse response associated with exposure to a substance.

IN USE describe suspected hypersensitivity

reaction history

The account of a previous response to a medicine or substance.

IN USE obtain a reaction history

suspected culprit

The medicine or substance thought possibly responsible.

IN USE identify the suspected culprit

generic name

The nonproprietary name of a medicine's active substance.

IN USE verify the generic name

brand name

A product name assigned by a manufacturer.

IN USE record the brand name

formulation

The prepared form of a medicine, such as a tablet or liquid.

IN USE confirm the formulation

route of administration

The way a medicine enters the body.

IN USE verify the route of administration

temporal association

A relationship in timing that does not alone prove causation.

IN USE describe a temporal association

Precision check

A report of a rash after a medicine establishes a reported sequence, not a confirmed immune mechanism. Preserve the report and seek appropriate review. No known allergies and information not established are different statuses; do not substitute one for the other.

The vocabulary that does the work

B Find the words / 2 of 2

03

Read the definitions and say the common word combinations aloud.

medication reconciliation

Comparing medication information to identify and resolve differences.

IN USE complete medication reconciliation

medication administration record

The record used to document medicines administered.

IN USE check the medication administration record

discrepancy

A difference between two accounts or records.

IN USE escalate a discrepancy

patient account

Information reported by the person receiving care.

IN USE preserve the patient account

corroborating record

Another reliable record that may support or clarify an account.

IN USE seek a corroborating record

unable to ascertain

Not possible to establish from the available information.

IN USE document information as unable to ascertain

no known drug allergies

A stated absence of known drug allergies, not a synonym for unknown.

IN USE verify a no-known-drug-allergies entry

allergy alert

A visible notification drawing attention to relevant allergy information.

IN USE review an allergy alert

contraindication

A circumstance making a treatment inappropriate under applicable clinical criteria.

IN USE check a contraindication

prescriber clarification

Information sought from the authorized prescribing professional.

IN USE request prescriber clarification

pharmacist review

Professional assessment of a medication-related question by a pharmacist.

IN USE obtain pharmacist review

record reconciliation

Resolving documented differences through an authorized process.

IN USE track record reconciliation

Precision check

Medication administration record is often shortened to MAR. Use approved terminology and documentation procedures locally. An alert, an order, and a pharmacist review serve different purposes; none should be treated as permission to guess away conflicting information.

Phrases you can actually use

C Notice the language / 1 of 2

03

STATE THE DISCREPANCY

"The allergy field and the patient's account do not match."

IDENTIFY THE UNKNOWN

"She cannot recall the drug name or exact date."

SEPARATE THE TIMING

"No current reaction has been reported in this scenario."

PRESERVE THE REPORT

"Do not erase the patient's account to make the fields agree."

ATTRIBUTE THE HISTORY

"Rosa reports a rash after an antibiotic several years ago."

AVOID DIAGNOSIS

"The cause of that historical reaction is not established."

SEEK REVIEW

"Please review this discrepancy through the medication-safety process."

AVOID FALSE CERTAINTY

"Unknown is not the same as no known allergies."

After versus because of

The first states a time relationship; the second claims causation.

Allergy versus intolerance

Do not assign either label solely to simplify the conversation.

None known versus unknown

One is a stated status; the other describes missing information.

Phrases you can actually use

C Notice the language / 2 of 2

03

ASK SPECIFICALLY

"What happened, when did it happen, and which medicine was involved?"

DISTINGUISH NAMES

"We need to verify the active substance, not assume from a remembered brand."

EXPLAIN THE PURPOSE

"I am checking the difference so the team has accurate information."

UPDATE APPROPRIATELY

"Record clarified information through the authorized documentation process."

CHECK OTHER RECORDS

"Is there an appropriate prior record that could clarify the medicine?"

ROUTE THE DECISION

"The medication decision needs the appropriately authorized clinical review."

CONFIRM RECEIPT

"Please confirm who is reviewing the unresolved history."

CLOSE THE DISCREPANCY

"Keep the difference visible until the review actually resolves it."

Historical versus current

A past report and a current reaction require distinct descriptions.

Reported

This word attributes information without dismissing it.

Reconciled

Use only when the relevant difference has actually been addressed.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which message best supports review?

- A** The record is complete because the patient cannot remember a date.
- B** Rosa is currently having the same reaction.
- C** The record says no known drug allergies; Rosa reports a past rash after an unnamed antibiotic.
- D** Rosa has a confirmed allergy to every antibiotic.

2. Which inference is unsupported?

- A** The record and patient account differ.
- B** The rash was definitely caused by an immune-mediated drug allergy.
- C** A historical rash was reported.
- D** The exact drug is not recalled.

3. What should happen to the unresolved information?

- A** Delete it because it is incomplete.
- B** Convert it to no known allergies without review.
- C** Mark it resolved because a message was sent.
- D** Keep it visible and route it for appropriate clinical clarification.

4. Which question seeks a useful missing detail without suggesting the answer?

- A** You meant stomach upset rather than a rash, correct?
- B** Do you remember the medicine's name or have a prior record we can appropriately check?
- C** It was definitely the same medicine as today's order, wasn't it?
- D** Can I assume the rash was unrelated?

Answers and explanations: [page 80](#).

The allergy field and the patient disagree

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

corroborating record / discrepancy / allergy status / Unable to ascertain / prescriber clarification / temporal association / Record reconciliation / pharmacist review / generic name / patient account

- 01 **Mei:** Sam, I need a medication-safety review. Rosa's record says no known drug allergies, but she reports a rash after an antibiotic several years ago.
- 02 **Sam:** I have the ¹ _____. Before we treat that field as settled, tell me exactly what Rosa said and which details you have not been able to establish.
- 03 **Mei:** She remembers a rash, but not the antibiotic name or the exact date. She is describing a past event; she has not reported a current reaction.
- 04 **Sam:** Keep that as the ² _____. We should not replace her words with a confirmed allergy diagnosis, but the missing details are not a reason to discard the report.
- 05 **Mei:** The old entry might have been copied forward. I cannot tell from this screen when somebody last checked it with her, so I should not call it verified.
- 06 **Sam:** Correct. We need to review the ³ _____ and the relevant history. An existing field is useful information, not proof that every later report has already been assessed.
- 07 **Mei:** Should I say that the antibiotic caused the rash? Rosa remembers taking it before the rash, but she does not have a diagnosis or written explanation.
- 08 **Sam:** Describe the ⁴ _____ without asserting the cause. We can say the rash followed an antibiotic in her account while leaving clinical interpretation to the appropriate review.
- 09 **Mei:** I can ask whether she remembers a product name or has a previous clinic record. I will not suggest a medicine and ask her to agree.
- 10 **Sam:** A ⁵ _____ could help. Use the appropriate access process and distinguish what the record actually says from what we still need to verify.

The conversation, continued

E Conversations / turns 11-20

03

Continue with the word bank on page 28.

- 11 **Mei:** If she recalls only a brand, we need to confirm the actual medicine rather than assume that every similarly named product contains the same substance.
- 12 **Sam:** Verify the ⁶ _____ and any relevant product details. That clarification belongs with the clinical review, not a guess made to complete an empty field.
- 13 **Mei:** For now, the drug name and date remain unknown. I should not select no known allergies just because the software encourages a completed status.
- 14 **Sam:** Exactly. ⁷ _____ and none known have different meanings. Follow our authorized documentation process while keeping the reported history and the unresolved details visible.
- 15 **Mei:** I will make clear that this is a historical report with no current reaction reported. That distinction should travel with the request, not disappear in the subject line.
- 16 **Sam:** I will take the ⁸ _____ and coordinate any necessary clarification with the appropriate clinician. This exchange itself does not authorize a medicine choice or administration decision.
- 17 **Mei:** If the prescriber needs to clarify the order in light of the history, we should receive the actual response rather than infer it from silence.
- 18 **Sam:** We will request ⁹ _____ through the correct route where needed. Record what was asked and what was answered, including any issue that remains open.
- 19 **Mei:** I will keep the discrepancy open until the relevant review is complete and the information is appropriately updated. Sending the request is not the final step.
- 20 **Sam:** Agreed. ¹⁰ _____ should reflect what the review establishes. Rosa's report, the old entry, and the eventual clarification each need an accurate place in that process.

ANSWERS / EXPLANATIONS: PAGE 81

1. discrepancy | 2. patient account | 3. allergy status | 4. temporal association | 5. corroborating record | 6. generic name | 7. Unable to ascertain | 8. pharmacist review | 9. prescriber clarification | 10. Record reconciliation

Unknown does not mean absent

F Say it

A patient remembers a previous reaction but cannot name the medicine. A new staff member proposes recording no known allergies solely because the name is unavailable.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

report / unknown / absent / clarification

Nurse: "The medicine name is ¹ _____."

Colleague: "That does not prove the history is ² _____."

Nurse: "Preserve the patient's ³ _____."

Colleague: "Route the discrepancy for qualified ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 81

1. unknown | 2. absent | 3. report | 4. clarification

Rehearse with a fixed script

1. Check your answers, then read turns 1-10 with a partner using the corrected words.
2. Switch roles for turns 11-20. Stress words that distinguish facts from conditions or estimates.
3. Complete and check the four-line exchange. Read it twice, switching roles and keeping the printed facts.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

A nod hides the wrong day

A Read the situation

Check understanding of supplied instructions, correct a misunderstanding without blame, and verify the corrected plan.

In a role-play, nurse Leila reviews an approved discharge sheet with patient Victor. Its appointment section says Thursday at 10:30 a.m., reception B, and bring the current medication list. The printed clinic number is for appointment questions, not an emergency route. Victor nods, then says he will return Friday. Leila must check and clarify the supplied appointment details without adding medical instructions. Victor may use the sheet while explaining the plan in his own words.

PEOPLE / Leila: Nurse | Victor: Patient

A NOD IS NOT A COMPLETED CHECK

Nodding may show attention, politeness, or a wish not to interrupt. Invite the patient to describe the plan without making the exchange sound like an examination. Take responsibility for an unclear explanation and check the corrected detail again.

Briefing checks

Circle one answer in each item. Use the briefing.

1. Which appointment details are supplied?

A Thursday, 10:30 a.m., reception B

C Thursday, 3:30 p.m., reception A

B Friday, 10:30 a.m., reception B

D Friday, 3:30 p.m., reception A

2. What does Victor's Friday answer reveal?

A Deliberate refusal to attend

C Proof that the written sheet is obsolete

B A confirmed change to the appointment

D A misunderstanding that needs clarification

3. What is the printed clinic number for in this case?

A Confirmation that no clinical follow-up is needed

C Any emergency instead of the applicable emergency route

B Appointment questions

D Direct permission to change medication

Answers and explanations: page 82.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

teach-back

Asking a person to explain information in their own words to check the explanation.

IN USE use teach-back

plain language

Familiar wording that communicates the intended meaning clearly.

IN USE explain in plain language

health literacy

The ability to find, understand, and use health information and services.

IN USE support health literacy

chunk and check

Giving small amounts of information and checking each part.

IN USE use chunk and check

discharge instructions

The approved information about the plan after leaving a care setting.

IN USE review discharge instructions

follow-up appointment

A planned later visit to continue review or care.

IN USE confirm the follow-up appointment

appointment window

A stated period for attending or arranging a visit.

IN USE clarify the appointment window

medication list

A record of medicines the person is using.

IN USE bring a current medication list

self-management

The person's role in carrying out an agreed care plan.

IN USE support self-management

caregiver involvement

Participation of a supporting person with appropriate permission.

IN USE confirm caregiver involvement

language access

Arrangements enabling meaningful communication across language differences.

IN USE arrange language access

qualified interpreter

A person with appropriate interpreting competence for the setting.

IN USE request a qualified interpreter

Precision check

Teach-back checks how clearly information was explained. It is not a memory test or a request to repeat every word exactly. A person may consult the approved sheet while describing the plan. A yes answer alone does not verify understanding.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

comprehension check

A focused check of what information has been understood.

IN USE make a comprehension check

misunderstanding

A difference between the intended meaning and the received meaning.

IN USE clarify a misunderstanding

rephrasing

Expressing the same meaning in different words.

IN USE try a clearer rephrasing

demonstration

Showing how a specified task is performed.

IN USE provide a demonstration

return demonstration

The learner showing a taught task back to the professional.

IN USE observe a return demonstration

action step

A specific part of the agreed plan to carry out.

IN USE identify the next action step

contact pathway

The appropriate route for a particular question or concern.

IN USE clarify the contact pathway

access barrier

A practical difficulty affecting use of information or services.

IN USE identify an access barrier

written reinforcement

Printed or digital information supporting the spoken explanation.

IN USE provide written reinforcement

nonjudgmental wording

Language that avoids blame or disparagement.

IN USE use nonjudgmental wording

understanding verified

A status supported by an actual check of the relevant information.

IN USE document understanding verified

education record

Documentation of information given and the person's demonstrated understanding.

IN USE update the education record

Precision check

A return demonstration checks a practical task, while this case checks appointment information. Do not invent clinical instructions to make the exercise more detailed. Keep routine appointment contacts distinct from the appropriate route for urgent clinical concerns.

Phrases you can actually use

C Notice the language / 1 of 2

INVITE THE PLAN

"In your own words, when and where will you return?"

CORRECT ONE DETAIL

"The appointment is Thursday, not Friday."

ALLOW THE REFERENCE

"You can keep the sheet in front of you."

CHECK LOCATION

"Which reception will you go to?"

TAKE RESPONSIBILITY

"I may not have made the day clear."

POINT TO THE SOURCE

"Let us look at the appointment line together."

CLARIFY THE TIME

"Ten thirty here means 10:30 in the morning."

CONFIRM WHAT TO BRING

"Please bring the current medication list shown on the sheet."

Do you understand?

A yes answer can conceal a misunderstanding; ask for the supplied plan in the patient's own words.

You got it wrong

Replace blame with a clear correction and responsibility for the explanation.

Repeat versus explain

Verbatim repetition may not show that the meaning is understood.

Phrases you can actually use

C Notice the language / 2 of 2

CHECK AGAIN

"Let us go over that part once more."

INVITE QUESTIONS

"What would you like me to explain more clearly?"

CHECK PRACTICAL ACCESS

"Is there any difficulty reaching that reception at the agreed time?"

ACKNOWLEDGE THE CORRECTION

"Yes, that matches the appointment details on the sheet."

AVOID A MEMORY TEST

"I am checking my explanation, not testing your memory."

SEPARATE CONTACT PURPOSES

"This clinic number is for appointment questions."

ARRANGE LANGUAGE SUPPORT

"We can arrange appropriate language assistance."

RECORD ACCURATELY

"Document the corrected plan you actually checked with the patient."

Current list

Current means the up-to-date list, not automatically any old printed version.

One number, one purpose

Do not broaden an appointment number into an emergency contact.

Check again

A correction is incomplete until the relevant point has been checked after rephrasing.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which question checks the supplied information most effectively?

- A** In your own words, when and where will you return, and what will you bring?
- B** Do you understand everything?
- C** You can read the sheet, can't you?
- D** You will remember all of that, right?

2. Victor says Friday. Which response is best?

- A** Friday is close enough to Thursday.
- B** I will record understanding verified without asking again.
- C** I may not have made the day clear; the sheet says Thursday. Let us check that part again.
- D** You were nodding, so you must already know.

3. Which entry is supported after Victor accurately restates the corrected details?

- A** Patient will certainly attend regardless of barriers
- B** No further questions can arise
- C** Appointment day, time, reception, and medication-list instruction checked through teach-back
- D** All discharge information understood permanently

4. Victor asks whether the clinic number replaces urgent care routes. What preserves the brief?

- A** It proves no urgent concern can occur.
- B** It authorizes changing treatment without review.
- C** No; its stated purpose here is appointment questions, and urgent concerns use the appropriate clinical route.
- D** Yes, every number on a discharge sheet serves every purpose.

Answers and explanations: page 82.

A nod hides the wrong day

E Conversations / turns 1-10

04

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

contact / plan / access barrier / location / reception B / appointment / appointment questions / medication list / current / detail

- 01 **Leila:** We have reached the appointment section on your discharge sheet. Before we finish, I would like to check that I have explained those details clearly.
- 02 **Victor:** I nodded because I was following most of it, but there were several details together. Could you go through the ¹ _____ part once more?
- 03 **Leila:** Of course. The sheet says Thursday at ten thirty in the morning, at reception B. It also says to bring your current medication list.
- 04 **Victor:** I think I have it. I will return Friday at ten thirty and go to reception B with my ² _____.
- 05 **Leila:** I may not have made the day clear. It is Thursday, not Friday. Let us look at that line together so the spoken and written information match.
- 06 **Victor:** Yes, I see Thursday now. I connected it with a different appointment on Friday. Having the sheet in front of me helps with that ³ _____.
- 07 **Leila:** Keep it there. I am not testing your memory. In your own words, when will you come back, and which reception will you use?
- 08 **Victor:** I will come on Thursday at ten thirty in the morning. I will go to ⁴ _____, rather than the desk where I first arrived today.
- 09 **Leila:** Yes, that matches the sheet. What will you bring with you for the appointment? You can look at the same line while you explain it.
- 10 **Victor:** I will bring my ⁵ _____ medication list. I have an older printed list at home, so I should not assume that old copy describes everything accurately now.

The conversation, continued

E Conversations / turns 11-20

04

Continue with the word bank on page 37.

- 11 **Leila:** The instruction is to bring the current list. If you need help clarifying what you have, use the appropriate team contact rather than guess about medication changes.
- 12 **Victor:** Understood. I also see a clinic number. Is that the ⁶ _____ for questions about this appointment, such as finding reception B?
- 13 **Leila:** Yes, that is the purpose identified in this section. It is not a replacement for the appropriate urgent clinical route if an urgent concern arises.
- 14 **Victor:** So I should not use the appointment number as a substitute for urgent help. Its stated purpose is ⁷ _____, not every possible concern after leaving.
- 15 **Leila:** Correct. Returning to this visit, is there any practical difficulty reaching reception B at the time shown? I can help clarify the location information available here.
- 16 **Victor:** I was not sure whether B meant the building or the desk. Could you explain the ⁸ _____ on the clinic map rather than assume I know this site?
- 17 **Leila:** Certainly. We can check the clinic map together before you leave. I will not direct you from memory if the map or local arrangements need clarification.
- 18 **Victor:** Thank you. That would remove an ⁹ _____ for me. The day is now clear, and the next thing I need is the exact way to that reception.
- 19 **Leila:** We will clarify that together. Then I will record the appointment details we checked, rather than write a broad statement that every discharge instruction is understood.
- 20 **Victor:** Let me give the ¹⁰ _____ once more: Thursday, ten thirty in the morning, reception B, with my current medication list. We will check the map before I leave.

ANSWERS / EXPLANATIONS: PAGE 83

1. appointment | 2. medication list | 3. detail | 4. reception B | 5. current | 6. contact | 7. appointment questions | 8. location | 9. access barrier | 10. plan

Correct the location, then check it

F Say it

An approved appointment note says Tuesday at 2 p.m., reception C. A patient correctly repeats the day and time but says reception A. The nurse clarifies only the location.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

words / location / C / afternoon

Nurse: "The day and time match; let us clarify the ¹ _____."

Patient: "The note says reception ² _____."

Nurse: "Please explain that corrected detail in your own ³ _____."

Patient: "I will use reception C on Tuesday at two in the ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 83

1. location | 2. C | 3. words | 4. afternoon

Rehearse with a fixed script

1. Check your answers, then read turns 1-10 with a partner using the corrected words.
2. Switch roles for turns 11-20. Stress words that distinguish facts from conditions or estimates.
3. Complete and check the four-line exchange. Read it twice, switching roles and keeping the printed facts.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Walking the corridor is not testing the stairs

A Read the situation

Present a functional finding, challenge an overbroad discharge summary, and agree on the next professional review.

During simulated rounds, nurse Omar sees a proposed noon discharge and a note saying "mobilizing well." Physiotherapist Eva reports a 40-meter corridor walk with a wheeled walker and supervision. Stairs were not assessed. The stated home access includes 12 steps and no elevator. Eva has not cleared stair use or independent mobility. The team needs the specific finding and unresolved access question; the language exercise does not decide fitness for discharge or prescribe a mobility technique.

PEOPLE / Omar: Nurse | Eva: Physiotherapist

DISAGREE WITH THE INFERENCE, NOT THE PROFESSION

Different disciplines bring different observations to rounds. A functional concern is not automatically an objection to efficient discharge planning. Name the assessed task, the conditions, and the unassessed task so the team can decide from a shared picture rather than a vague label.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What was actually assessed?

- | | |
|--|---|
| A Independent stair climbing | B Twelve steps without equipment |
| C All activities required at home | D A 40-meter corridor walk with a wheeled walker and supervision |

2. What remains unresolved?

- | | |
|--|--|
| A Whether the home has any steps | B Whether a corridor walk occurred |
| C Whether the proposed time is noon | D Stair access to the stated home setting |

3. What is the team's immediate communication need?

- | | |
|---|---|
| A Removal of the supervision condition to shorten the note | B A precise finding and a clear unresolved question for the appropriate review |
| C A guarantee that noon discharge is safe | D A mobility technique invented from the dialogue |

Answers and explanations: page 84.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

interprofessional rounds

A care discussion involving professionals from different disciplines.

IN USE contribute to interprofessional rounds

functional assessment

Evaluation of how a person performs relevant practical tasks.

IN USE report a functional assessment

ambulation

Walking or moving about on foot.

IN USE describe observed ambulation

gait

The pattern of walking.

IN USE assess gait

mobility aid

Equipment used to support movement.

IN USE specify the mobility aid

wheeled walker

A walking support with wheels.

IN USE use a wheeled walker

supervision

Oversight without assuming the same meaning as physical assistance.

IN USE document required supervision

physical assistance

Hands-on help provided during a task.

IN USE specify physical assistance

independent mobility

Movement without another person's assistance or supervision for the stated task.

IN USE verify independent mobility

transfer ability

Capacity to move between positions or surfaces.

IN USE assess transfer ability

stair negotiation

Managing ascent or descent of steps.

IN USE assess stair negotiation

activity tolerance

Capacity to sustain a specified activity.

IN USE describe activity tolerance

Precision check

A distance alone does not describe functional performance. Include the aid, assistance or supervision, setting, and assessed task. A corridor walk does not establish stair ability. Not assessed is different from assessed and unable; preserve that distinction.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

home environment

The physical and practical conditions in the person's home setting.

IN USE assess the home environment

access requirement

A condition that must be met to enter or use a setting.

IN USE clarify access requirements

discharge readiness

Whether relevant conditions for leaving the care setting have been met.

IN USE review discharge readiness

discharge destination

The setting to which a person is expected to leave.

IN USE confirm the discharge destination

equipment provision

Arranging necessary equipment through the appropriate process.

IN USE verify equipment provision

functional limitation

A restriction in performing a specified task.

IN USE describe a functional limitation

unassessed task

A relevant activity not yet evaluated.

IN USE identify an unassessed task

discipline-specific finding

An observation within a professional field's assessment.

IN USE share discipline-specific findings

shared goal

An agreed outcome supported by the care team and patient.

IN USE establish a shared goal

discharge barrier

An unresolved issue relevant to a proposed departure.

IN USE communicate a discharge barrier

therapy recommendation

Advice from the relevant therapy assessment.

IN USE clarify the therapy recommendation

team decision

A conclusion reached through the appropriate care-team process.

IN USE record the team decision

Precision check

Physiotherapist and physical therapist are common regional titles. Assistance labels vary across settings, so use the local definitions. A proposed departure time does not override unresolved clinical or functional questions, and a language exercise cannot supply a discharge clearance.

Phrases you can actually use

C Notice the language / 1 of 2

STATE THE FINDING

"Eva observed a 40-meter corridor walk."

INCLUDE THE CONDITIONS

"The walk used a wheeled walker and required supervision."

NAME THE GAP

"Stairs have not been assessed."

CLARIFY THE HOME

"The stated home access has twelve steps and no elevator."

CHALLENGE THE SUMMARY

"Mobilizing well is too broad for the finding we have."

AVOID A FALSE NEGATIVE

"Not assessed does not mean unable."

AVOID A FALSE POSITIVE

"Corridor performance does not establish stair safety."

PRESERVE PROFESSIONAL SCOPE

"The relevant qualified team must review the discharge implications."

Mobilizing well

Too vague without the task, conditions, and relevant limits.

Independent

Do not use this term when supervision was required in the supplied assessment.

Not assessed

A statement about evidence, not a prediction of success or failure.

Phrases you can actually use

C Notice the language / 2 of 2

SEPARATE TIMING FROM READINESS

"Noon is a proposed time, not evidence that the conditions are met."

STATE A SHARED GOAL

"We want a workable plan that reflects the actual home setting."

MAKE THE CONCERN SPECIFIC

"My concern is the unassessed task required at the destination."

CLARIFY THE RECORD

"Record the observed task separately from the outstanding assessment."

INVITE THE NEXT REVIEW

"What assessment is needed to address the stair-access question?"

CHECK EQUIPMENT

"Confirm the equipment arrangements rather than assume they are complete."

PREVENT OVERSTATEMENT

"Please retain the supervision condition in the summary."

CLOSE ROUNDS ACCURATELY

"The discharge decision remains with the appropriate clinical process."

Cleared

Specify who made which decision and on what basis.

Home today

A proposed destination and date are not a completed readiness assessment.

Common goal

Shared discharge planning can include respectful disagreement about missing evidence.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which summary is most accurate?

- A** Unable to climb stairs, as demonstrated today.
- B** Cleared for twelve steps because the corridor distance was longer.
- C** Walked 40 meters with a wheeled walker and supervision; stairs not assessed.
- D** Independently mobile in all home activities.

2. Why does the home information matter?

- A** An elevator was confirmed available.
- B** Home access never affects discharge planning.
- C** The destination requires a task that the corridor assessment did not establish.
- D** Twelve steps are always equivalent to forty meters of walking.

3. How should Omar respond to pressure for a noon summary?

- A** Treat the proposed time as the assessment result.
- B** Report the specific assessment and unresolved stair question for the qualified team.
- C** Remove supervision to make the note more decisive.
- D** Convert not assessed to safe.

4. Which statement incorrectly treats missing evidence as a negative finding?

- A** Eva has not assessed stairs.
- B** The corridor walk required supervision.
- C** The home access includes twelve steps.
- D** Eva established that the patient cannot use stairs.

Answers and explanations: [page 84](#).

Walking the corridor is not testing the stairs

E Conversations / turns 1-10

05

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

discharge readiness / functional assessment / shared goal / team decision / unassessed task / equipment provision / discharge destination / supervision / home environment / stair negotiation

- 01 **Omar:** The proposed departure time is noon, and the rounds note says mobilizing well. Before I repeat that to the team, can you tell me exactly what you assessed?
- 02 **Eva:** My ¹ _____ included a forty-meter corridor walk. The patient used a wheeled walker and required supervision. Stairs were not part of that assessment.
- 03 **Omar:** The home information says twelve steps and no elevator. I was about to describe the patient as independently mobile, but that would leave out your conditions.
- 04 **Eva:** Please retain ² _____. The distance alone does not establish independence. The equipment, setting, and level of support are part of the finding, not optional details.
- 05 **Omar:** Some colleagues may hear that as a general objection to discharge. I need to make the unresolved point specific, so rounds can address it efficiently.
- 06 **Eva:** The unresolved question is ³ _____ in relation to the stated home access. I am not saying the patient failed a stair assessment; I am saying it has not occurred.
- 07 **Omar:** That distinction matters. If I write unable to manage steps, I would be turning an absence of assessment into a result that you never reported.
- 08 **Eva:** Correct. It is an ⁴ _____. Neither safe on stairs nor unable on stairs follows from the corridor walk. We need the relevant professional review.
- 09 **Omar:** I will tell the team that the home access requires twelve steps. The proposed noon time should be discussed alongside that fact rather than erase it.
- 10 **Eva:** The ⁵ _____ gives the assessment its practical context. A finding can be accurate for this corridor and still leave a question about the destination unanswered.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 46.

05

- 11 **Omar:** We also need to avoid assuming the walker arrangements are complete. The assessment tells us what was used here, not what has been arranged elsewhere.
- 12 **Eva:** Yes, ⁶ _____ is a separate status to verify through the appropriate team. We should not turn observed use into confirmation that every arrangement is finished.
- 13 **Omar:** How should I summarize this at rounds without asking everyone to read a long therapy note? I want one concise statement and the unresolved question.
- 14 **Eva:** State the observed walk and support, then the stair question affecting ⁷ _____. Keep the decision with the appropriate clinical process rather than presenting my observation as complete clearance.
- 15 **Omar:** I can say: forty meters with a wheeled walker and supervision; stairs not assessed; home access includes twelve steps without an elevator. The stair question needs review.
- 16 **Eva:** That is accurate. Our ⁸ _____ is a workable plan for the actual destination, not simply a shorter note or an argument about which discipline controls the schedule.
- 17 **Omar:** If the team changes the destination or obtains another assessment, we should update the summary from those actual facts rather than predict the result now.
- 18 **Eva:** Exactly. Confirm the ⁹ _____ and relevant findings as they are established. A different setting may raise different questions, but we cannot invent that change in advance.
- 19 **Omar:** I will replace mobilizing well with the specific finding and bring the outstanding question to rounds. Noon remains proposed, not clinically confirmed by this exchange.
- 20 **Eva:** Thank you. Record the eventual ¹⁰ _____ through the appropriate process, including any conditions. My contribution is the assessment actually performed and the limit of what it establishes.

ANSWERS / EXPLANATIONS: PAGE 85

1. functional assessment | 2. supervision | 3. stair negotiation | 4. unassessed task | 5. home environment | 6. equipment provision | 7. discharge readiness | 8. shared goal | 9. discharge destination | 10. team decision

Measured distance, untested task

F Say it

A therapist observes a 25-meter indoor walk with assistance. Outdoor uneven ground has not been assessed. A summary incorrectly calls the person independent outdoors.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

unassessed / independence / indoors / assistance

Therapist: "The observed walk was ¹ _____."

Nurse: "It required ² _____."

Therapist: "Uneven outdoor ground is ³ _____."

Nurse: "The summary must not claim outdoor ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 85

1. indoors | 2. assistance | 3. unassessed | 4. independence

Rehearse with a fixed script

1. Check your answers, then read turns 1-10 with a partner using the corrected words.
2. Switch roles for turns 11-20. Stress words that distinguish facts from conditions or estimates.
3. Complete and check the four-line exchange. Read it twice, switching roles and keeping the printed facts.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Replace the label with the observable facts

A Read the situation

Select an objective, attributed clinical note and distinguish event time, entry time, and a later correction.

06

In a documentation simulation, nurse Elena drafts "Patient difficult and refusing all care." At 2:20 p.m., the patient said, "I want to rest before washing." Elena offered to return at 3 p.m.; the patient agreed. No other care was discussed. Elena enters the note at 2:45 p.m. Supervisor Chris reviews its wording. No capacity assessment, diagnosis, or explanation beyond the quoted request is supplied.

PEOPLE / Elena: Nurse | Chris: Supervisor

A USEFUL NOTE CAN BE BOTH PRECISE AND RESPECTFUL

Labels may feel efficient, but they can hide the event that the next clinician needs to understand. Record observable behavior, attributed words, relevant actions, and timing. Neutral wording does not require deleting a difficult interaction; it requires describing it accurately.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What did the patient actually request?

- A** A confirmed change in diagnosis
- B** Rest before washing
- C** Refusal of every type of care
- D** Permanent cancellation of all support

2. Which times need to be distinguished?

- A** Event at 2:45 p.m. and entry at 2:20 p.m.
- B** Only the computer's current date
- C** Event at 2:20 p.m. and entry at 2:45 p.m.
- D** Event and entry both at 3 p.m.

3. What was agreed?

- A** The patient had already washed at 3 p.m.
- B** No further contact would occur
- C** Every future care offer would be declined
- D** Elena would return at 3 p.m.

Answers and explanations: [page 86](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

objective documentation

Recording observable or verifiable information without unsupported judgment.

IN USE use objective documentation

subjective report

Information about an experience described by the person.

IN USE attribute a subjective report

direct quotation

The person's exact words identified as a quote.

IN USE include a relevant direct quotation

behavioral description

A factual account of what a person said or did.

IN USE provide a behavioral description

clinical inference

A professional interpretation distinguished from the underlying facts.

IN USE separate clinical inference from observation

event time

The time the described action or observation occurred.

IN USE record the event time

entry time

The time the documentation is entered.

IN USE preserve the entry time

late entry

A later record of an earlier event, identified according to local procedure.

IN USE identify a late entry

addendum

Additional information appended to an existing record.

IN USE make an authorized addendum

amendment

A documented correction or change made through the approved process.

IN USE request an amendment

audit trail

A record showing relevant entries, changes, authors, and times.

IN USE preserve the audit trail

record integrity

The reliability and traceability of documented information.

IN USE protect record integrity

Precision check

The quoted request supports a preference about timing, not refusal of all care or a capacity finding. Record the offered return and the agreement. Do not document a planned action as completed before it occurs.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

copy-forward

Reusing earlier text in a later record.

IN USE verify copy-forward content

template text

Preset wording inserted into a record.

IN USE review template text

autopopulated field

A field filled automatically from another source.

IN USE verify autopopulated fields

author attribution

Identification of the person responsible for an entry.

IN USE maintain author attribution

contemporaneous note

Documentation made at or close to the relevant event.

IN USE make a contemporaneous note

factual correction

A change that corrects an inaccurate statement.

IN USE document a factual correction

care declined

A specific offered action that the person did not accept.

IN USE specify the care declined

agreed follow-up

The next contact or action actually agreed with the person.

IN USE record agreed follow-up

capacity assessment

A qualified assessment of decision-making ability under applicable criteria.

IN USE document a capacity assessment accurately

informed refusal

Declining an intervention after the relevant information and process.

IN USE distinguish an informed refusal

approved abbreviation

A shortened form permitted and understood in the local documentation system.

IN USE use approved abbreviations

documentation scope

The limits of what an entry can support from the available evidence.

IN USE keep within documentation scope

Precision check

Event time and entry time answer different questions. Follow local rules for delayed entries and corrections; never disguise when a note was entered. A preserved audit trail is compatible with correcting an inaccurate statement through the authorized process.

Phrases you can actually use

C Notice the language / 1 of 2

REPLACE THE LABEL

"Describe what was said and what you observed."

LIMIT THE CLAIM

"No other care was discussed in this interaction."

RECORD AGREEMENT

"The patient agreed to that return time."

AVOID AN INFERRED MOTIVE

"The record does not establish why the patient wanted to rest beyond the stated request."

ATTRIBUTE THE WORDS

"The patient said, 'I want to rest before washing.'"

RECORD THE OFFER

"I offered to return at 3 p.m."

SEPARATE THE TIMES

"The event occurred at 2:20; this entry is being made at 2:45."

KEEP PLANS PROSPECTIVE

"A planned return is not a completed visit."

06

Difficult

A label that does not identify an observable action.

Refusing all care

Far broader than a request to postpone one task.

Patient stated

Attributes the account rather than presenting it as your independent finding.

Phrases you can actually use

C Notice the language / 2 of 2

CHECK COPIED WORDING

"That phrase came from an earlier note and needs verification."

CORRECT TRANSPARENTLY

"Use the approved amendment process rather than conceal the earlier entry."

USE RELEVANT DETAIL

"Include the facts needed for the next person's follow-up."

CHECK THE FINAL NOTE

"Does every statement have a source in the supplied facts?"

PRESERVE THE AUTHOR

"Attribute the entry to the person making it."

AVOID OVERREACH

"This interaction does not establish a capacity assessment."

CLARIFY AN ABBREVIATION

"Use the approved term where a shortened form could be misunderstood."

CLOSE ACCURATELY

"Record the later return only after its actual outcome is known."

06

Agreed versus completed

An agreement concerns a plan; completion concerns an event that occurred.

Backdating

Do not disguise the actual time of entry.

Correction versus concealment

Correct errors through the approved process while preserving appropriate traceability.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which replacement note best matches the event?

- A** At 2:20, patient said, "I want to rest before washing." Return at 3 offered and agreed.
- B** Patient refused all clinical care throughout the day.
- C** Patient lacked capacity because washing was postponed.
- D** Patient was uncooperative for reasons not recorded.

2. Which claim is not supported?

- A** The entry was made after the event.
- B** The patient refused every type of care.
- C** The patient requested rest before washing.
- D** Elena offered to return at three.

3. How should the two times be handled?

- A** Change the entry time to make it appear contemporaneous.
- B** Omit the event time and assume readers will infer it.
- C** Record the event at three because that was the planned return.
- D** Identify the event time and preserve the actual entry time under local procedure.

4. At 2:45, which follow-up statement is accurate?

- A** All subsequent care was accepted.
- B** Return at 3 p.m. agreed; outcome not yet recorded.
- C** Returned at 3 and completed washing.
- D** Patient later refused the visit.

Answers and explanations: [page 86](#).

Replace the label with the observable facts

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

06

WORD + PHRASE BANK

event time / amendment / clinical inference / documentation scope / agreed follow-up / direct quotation / Objective documentation / template text / audit trail / behavioral description

- 01 **Elena:** My draft says the patient was difficult and refusing all care. Reading it back, I can see that it does not explain what happened during the interaction.
- 02 **Chris:** Start with a ¹ _____. What did the patient actually say, what did you offer, and which part of care was discussed?
- 03 **Elena:** At two twenty, the patient said, "I want to rest before washing." I offered to return at three, and the patient agreed to that time.
- 04 **Chris:** That ² _____ is useful because it states the request precisely. It does not support a claim that every kind of care was refused.
- 05 **Elena:** No other care was discussed. I should not make the note broader just because the original template gave me a short phrase about noncompliance.
- 06 **Chris:** Correct. Review the ³ _____ rather than let it determine the event. A convenient field does not justify recording something that the interaction did not establish.
- 07 **Elena:** I am entering the note at two forty-five. The conversation happened at two twenty, so those times describe different things even though they are close together.
- 08 **Chris:** Preserve the ⁴ _____ and the actual entry time according to our documentation procedure. Do not make the entry appear to have been written earlier than it was.
- 09 **Elena:** I also wrote that I returned at three, but that is still in the future. I meant that I had arranged to return then.
- 10 **Chris:** Record it as ⁵ _____. The patient agreed to your offer, but the outcome of that future visit cannot be documented as complete now.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 55.

- 11 **Elena:** Should I include a reason such as the patient being anxious? I thought that might explain why the patient wanted to delay washing.
- 12 **Chris:** Only add a supported ⁶ _____ within your role, clearly distinguished from the facts. The account supports a request for rest, not an established finding of anxiety or another explanation.
- 13 **Elena:** The phrase refusing all care may already have been saved in the draft record. I should check its status rather than silently assume nobody could have seen it.
- 14 **Chris:** Follow the authorized ⁷ _____ process if a saved entry needs correction. Keep the change traceable and do not obscure the original entry or its author.
- 15 **Elena:** So I should not delete the history merely to make the final record look as though the inaccurate wording never existed.
- 16 **Chris:** Preserve the ⁸ _____ in the appropriate system. Transparent correction helps a later reader understand what was changed and why the current wording is more accurate.
- 17 **Elena:** The final note can state the quotation, the offer to return, and the agreement. It should not imply a formal decision about capacity or informed refusal.
- 18 **Chris:** That respects the ⁹ _____. Those are distinct clinical and procedural matters, and the facts here do not establish that either assessment occurred.
- 19 **Elena:** I will check each sentence against the actual interaction. At three, the next entry will describe what really happens, including any further agreement or change.
- 20 **Chris:** Good. ¹⁰ _____ can remain respectful without losing detail. The next clinician should understand the specific request and follow-up, rather than inherit a label about the patient.

06

ANSWERS / EXPLANATIONS: PAGE 87

1. behavioral description | 2. direct quotation | 3. template text | 4. event time | 5. agreed follow-up | 6. clinical inference | 7. amendment | 8. audit trail | 9. documentation scope | 10. Objective documentation

A plan entered as an event

F Say it

At 11 a.m., a note says "Education completed at noon." The only supplied fact is an agreement to return at noon for education. The noon visit has not occurred.

Complete the four gaps. Use each bank entry once.

06

WORD + PHRASE BANK

outcome / future / process / planned

Reviewer: "Noon is still in the ¹ _____."

Author: "The visit is ² _____, not completed."

Reviewer: "Correct the statement through the approved ³ _____."

Author: "The later entry must reflect the actual ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 87

1. future | 2. planned | 3. process | 4. outcome

Rehearse with a fixed script

1. Check your answers, then read turns 1-10 with a partner using the corrected words.
2. Switch roles for turns 11-20. Stress words that distinguish facts from conditions or estimates.
3. Complete and check the four-line exchange. Read it twice, switching roles and keeping the printed facts.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Acknowledge the frustration; keep the boundary

A Read the situation

Respond to a family member with empathy, accurate limits, and a concrete next contact without promising another clinician.

In a simulation, family member Anika says, "Nobody tells us anything," and asks nurse Luis to explain a test result. The patient has agreed to include Anika in relevant updates; that permission does not authorize Luis to invent an interpretation. Luis can contact the responsible clinician and return with a status update by 4 p.m. No clinician meeting time is confirmed. Anika is frustrated but is not making threats. Urgent clinical concerns would follow the appropriate separate route.

PEOPLE / Anika: Family member | Luis: Nurse

07

BOUNDARIES SHOULD STILL OFFER A WAY FORWARD

A boundary can sound dismissive when it ends the conversation without an alternative. Acknowledge the specific concern, explain what you can do, and state the next contact. Do not describe every frustrated family member as aggressive or confuse a firm request with a threat.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What permission is supplied?

A Anika can make every decision for the patient.

B Luis can interpret any result without the appropriate role.

C Every relative may receive all records.

D The patient has agreed to include Anika in relevant updates.

2. What can Luis promise?

A His own status update by 4 p.m.

B A clinician meeting at exactly 4 p.m.

C A reassuring interpretation of the result

D Completion of every unresolved issue by four

3. How is Anika's behavior described?

A A confirmed reason to end all communication

B Frustrated, without threats

C Physically threatening

D Calm and satisfied

Answers and explanations: [page 88](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

family liaison

Communication connecting family members with the care team.

IN USE coordinate family liaison

patient preference

The person's expressed choice relevant to their care or communication.

IN USE respect patient preferences

permission to share

An appropriate basis allowing specified information to be communicated.

IN USE verify permission to share

confidentiality

Protection of information from inappropriate access or disclosure.

IN USE maintain confidentiality

authorized recipient

A person appropriately entitled to receive the relevant information.

IN USE confirm the authorized recipient

professional boundary

A limit that keeps actions and communication within an appropriate role.

IN USE maintain a professional boundary

scope of practice

The professional activities permitted by role, competence, and applicable rules.

IN USE work within scope of practice

empathic acknowledgment

Recognition of a person's concern or experience without unsupported agreement.

IN USE offer empathic acknowledgment

validation

Recognition that an expressed feeling or concern has been heard.

IN USE provide appropriate validation

de-escalation

Communication or action intended to reduce tension and support safety.

IN USE use de-escalation skills

active listening

Attentive listening that checks the speaker's meaning.

IN USE demonstrate active listening

clarifying question

A question used to establish the specific concern or missing detail.

IN USE ask a clarifying question

Precision check

Permission to include a relative in updates is not automatically authority to decide for the patient. A nurse can acknowledge frustration without confirming an unsupported allegation. A status update promises communication; a clinical explanation requires the appropriate professional involvement.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

family meeting

An arranged discussion involving appropriate relatives and care professionals.

IN USE request a family meeting

clinical explanation

Interpretation of clinical information by an appropriately qualified professional.

IN USE arrange a clinical explanation

status update

Information about what has happened and what remains pending.

IN USE provide a status update

update commitment

A specific promise to communicate again at an agreed point.

IN USE honor the update commitment

named contact

The identified person responsible for the next communication.

IN USE provide a named contact

expectation management

Clear communication about what can and cannot currently be promised.

IN USE improve expectation management

complaint pathway

The designated route for raising and reviewing service concerns.

IN USE explain the complaint pathway

shared decision-making

A process combining relevant evidence with the patient's values and preferences.

IN USE support shared decision-making

substitute decision-maker

A person with applicable authority to decide in specified circumstances.

IN USE verify substitute decision-maker authority

consent discussion

A relevant exchange supporting an appropriately informed decision.

IN USE arrange a consent discussion

communication preference

A stated choice about how or with whom information is shared.

IN USE record communication preferences

safety boundary

A limit or action needed to protect people from actual risk.

IN USE apply an appropriate safety boundary

Precision check

Frustration, assertiveness, threats, and violence are not interchangeable labels. Respond to the actual behavior and local safety procedures. This scenario supplies no threat; it calls for listening, accurate boundaries, and reliable follow-through rather than punitive language.

Phrases you can actually use

C Notice the language / 1 of 2

ACKNOWLEDGE THE EXPERIENCE

"You have been waiting for a clear explanation, and that is frustrating."

CONFIRM PARTICIPATION

"The patient has agreed that you can be included in these updates."

ARRANGE THE RIGHT DISCUSSION

"I will contact the responsible clinician about that question."

SEPARATE THE APPOINTMENT

"Four is my update time, not a confirmed meeting with the clinician."

CLARIFY THE CONCERN

"Which question has not yet been answered?"

KEEP THE ROLE CLEAR

"I can explain the next step, but I cannot invent an interpretation of the result."

PROMISE WHAT YOU CONTROL

"I will return with a status update by 4 p.m."

AVOID UNSUPPORTED REASSURANCE

"I cannot tell you the result is normal from the information we have here."

I understand

Name what you understand rather than use the phrase as a substitute for listening.

Nobody

A frustrated absolute can be acknowledged without debating every previous contact.

I promise

Restrict the promise to an action you can actually control.

Phrases you can actually use

C Notice the language / 2 of 2

KEEP LISTENING

"Let me check that I have understood your main concern."

CLARIFY AUTHORITY

"Being included in updates does not by itself determine decision-making authority."

EXPLAIN DELAY HONESTLY

"The meeting time is still unconfirmed, and I will tell you what I have established."

PROTECT URGENT RESPONSE

"A new urgent clinical concern needs the appropriate clinical route immediately."

AVOID DEFENSIVENESS

"I want to address the missing explanation rather than argue about your frustration."

NAME THE NEXT CONTACT

"I will be your contact for this promised update."

OFFER THE FORMAL ROUTE

"I can explain how to raise the communication concern through the appropriate pathway."

CLOSE WITH THE COMMITMENT

"I will update you by four even if the meeting time remains pending."

Normal

Do not use this as reassurance without an established clinical basis.

Included versus authorized to decide

These are different permissions and responsibilities.

Difficult family

Describe the actual communication issue rather than treating the label as an assessment.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which opening best acknowledges the concern?

- A** You are wrong because staff have spoken today.
- B** Calm down before anyone tells you anything.
- C** Every test result is probably fine.
- D** You have been waiting for a clear explanation; let me check the unanswered question and arrange the right contact.

2. Anika asks, "Will the clinician be here at four?" What is accurate?

- A** Four is my promised status update; a clinician meeting time is not yet confirmed.
- B** Yes, my update promise guarantees the clinician's attendance.
- C** No clinician will ever attend because no time is booked now.
- D** Four means the result will definitely be normal.

3. What should Luis avoid inferring from permission to share updates?

- A** That relevant updates may include Anika
- B** That patient preferences matter
- C** That the result question needs the right clinical professional
- D** That Anika automatically has full decision-making authority

4. At four, no meeting time is confirmed. What honors the commitment?

- A** Give Anika the current status and a clear next communication step.
- B** Say nothing until a meeting can be guaranteed.
- C** Invent a meeting time to reduce frustration.
- D** Mark the concern resolved because a message was sent.

Answers and explanations: [page 88](#).

Acknowledge the frustration; keep the boundary

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

concern / named contact / responsible clinician / reassurance / updates / status update / clear explanation / decision-making authority / update time / commitment

- 01 **Anika:** Nobody tells us anything. We keep hearing that someone will explain the result, but we still do not know who that person is or when we will hear.
- 02 **Luis:** You have been waiting for a ¹ _____, and I can hear how frustrating that is. Let me check which question you most need answered.
- 03 **Anika:** I want to know what the test result means for my father. I do not want another message saying that somebody has passed the request to somebody else.
- 04 **Luis:** The patient has agreed to include you in these ² _____. I can contact the responsible clinician with that question, but I should not invent a clinical interpretation.
- 05 **Anika:** Does that mean you cannot tell me anything? I am not asking for a lecture about whose job it is. I need a practical next step.
- 06 **Luis:** The next step is for me to contact the ³ _____ and confirm the response arrangements. I will stay responsible for the update I promise you.
- 07 **Anika:** All right, but what exactly are you promising? Earlier I heard soon, and soon turned into several hours without a clear answer.
- 08 **Luis:** I will give you a ⁴ _____ by four this afternoon. That will include what I have confirmed and anything still awaiting a response.
- 09 **Anika:** So the clinician will meet us at four? I can ask my sister to come then if that is a firm time.
- 10 **Luis:** Four is my ⁵ _____, not a confirmed clinician meeting. I do not want you to rearrange the family around an appointment that has not been agreed.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 64.

- 11 **Anika:** Thank you for making that distinction. I am still worried. Can you at least say the result is probably normal because nobody has come urgently?
- 12 **Luis:** I cannot give that ⁶ _____ from silence or timing. The result needs the appropriate clinical explanation, and I will carry that specific question to the responsible team.
- 13 **Anika:** I also want them to know that the lack of communication has made this harder. I should not have to become upset before somebody gives a clear answer.
- 14 **Luis:** I will include that ⁷ _____ accurately. If you want it reviewed through the formal communication-complaint route, I can explain that process as well.
- 15 **Anika:** I would like the explanation arranged first. Since my father agreed to include me, does that also mean I can make all the decisions for him?
- 16 **Luis:** Inclusion in updates does not by itself establish ⁸ _____. His preferences and the applicable decision process still matter. We should clarify any specific decision with the appropriate team.
- 17 **Anika:** That is fair. My immediate request is the result discussion and a reliable contact. I do not want to start the whole explanation again with every shift.
- 18 **Luis:** I will be the ⁹ _____ for this promised update and document the concern and next step appropriately. The team should receive the question you have already explained.
- 19 **Anika:** If the meeting time is still unknown at four, please tell me that directly. Waiting without knowing whether the request was received is the hardest part.
- 20 **Luis:** I will keep that ¹⁰ _____. You will hear the current status from me by four, with the next communication step, rather than an invented answer or an unconfirmed appointment.

07

ANSWERS / EXPLANATIONS: PAGE 89

1. clear explanation | 2. updates | 3. responsible clinician | 4. status update | 5. update time | 6. reassurance | 7. concern | 8. decision-making authority | 9. named contact | 10. commitment

An update is not a meeting

F Say it

A therapist promises a family a 5 p.m. update about arranging a care conference. The conference time remains unconfirmed. The patient has agreed to the relevant family involvement.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

commitment / status / time / update

Family member: "Is five the conference ¹ _____?"

Therapist: "Five is my ² _____; the conference is not yet booked."

Family member: "Please explain the current ³ _____ even if it is pending."

Therapist: "I will keep my communication ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 89

1. time | 2. update | 3. status | 4. commitment

Rehearse with a fixed script

1. Check your answers, then read turns 1-10 with a partner using the corrected words.
2. Switch roles for turns 11-20. Stress words that distinguish facts from conditions or estimates.
3. Complete and check the four-line exchange. Read it twice, switching roles and keeping the printed facts.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Report the fall without inventing its cause

A Read the situation

Separate an event chronology from causal assumptions and select a follow-up measure tied to the identified risk.

In a simulated safety review, a patient fall is recorded at 11:05 a.m. Immediate clinical response has followed local procedure; the review does not replace it. Nurse Beth did not witness the fall. The patient later reported standing to reach water. A draft says a colleague was careless and the call bell was out of reach, but neither claim is established. Safety lead Arun asks for the sources, sequence, and unresolved environmental details before conclusions or corrective actions are assigned.

PEOPLE / Beth: Nurse | Arun: Safety lead

08

FAIR REVIEW IS NOT AUTOMATIC BLAME OR AUTOMATIC ABSOLUTION

An event deserves a careful account of actions, context, and system conditions. A fair learning culture makes it possible to report concerns while retaining appropriate accountability. It does not establish a cause from an outcome or promise that every action is beyond review.

Briefing checks

Circle one answer in each item. Use the briefing.

1. Which event is supplied as recorded?

- A A verified unreachable call bell
- C A fall at 11:05 a.m.

- B A completed causal investigation
- D A proven act of carelessness

2. What is the source of the water detail?

- A The patient's later report
- C A confirmed equipment log

- B Beth's eyewitness account
- D A completed staff investigation

3. What is the purpose of this review?

- A Replace the immediate clinical response
- C Mark every suggested cause as verified

- B Assign blame from the outcome alone
- D Reconstruct supported facts and unresolved details before conclusions

Answers and explanations: page 90.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

patient safety event

An occurrence relevant to possible or actual harm during care.

IN USE report a patient safety event

event chronology

The ordered sequence of known events and times.

IN USE reconstruct the event chronology

firsthand observation

Information directly witnessed by the person reporting it.

IN USE distinguish firsthand observation

secondhand account

Information received from another person rather than directly witnessed.

IN USE attribute a secondhand account

contributing factor

A condition or action that helped an event occur.

IN USE investigate contributing factors

causal assumption

An explanation treated as true before sufficient support is established.

IN USE challenge a causal assumption

hindsight bias

Judging earlier actions as obviously wrong because the outcome is now known.

IN USE avoid hindsight bias

outcome bias

Judging a decision mainly by its result rather than the information available then.

IN USE recognize outcome bias

just culture

A fair approach combining learning from systems with appropriate individual accountability.

IN USE support a just culture

psychological safety

A climate in which people can raise concerns without interpersonal intimidation.

IN USE strengthen psychological safety

human factors

Study of interactions between people, tasks, equipment, and working conditions.

IN USE apply human-factors analysis

systems approach

Review of how connected processes and conditions affect performance.

IN USE use a systems approach

08

Precision check

A fall is an event; carelessness is an interpretation that requires evidence. Attribute the patient's account and distinguish it from firsthand observation. Absence of a witness does not make the report irrelevant, but it limits what the reporter can personally verify.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

environmental condition

A relevant physical feature of the setting.

IN USE verify environmental conditions

call-bell access

Whether the patient can reach and use the assistance-call device.

IN USE check call-bell access

immediate response

Actions taken promptly to address the situation and possible harm.

IN USE document the immediate response

incident report

The formal record submitted through a safety-reporting process.

IN USE complete an incident report

clinical record

The care documentation concerning the individual patient.

IN USE maintain the clinical record

evidence preservation

Protecting relevant records or information for accurate review.

IN USE support evidence preservation

root cause analysis

A structured investigation of underlying and contributing causes.

IN USE contribute to root cause analysis

corrective action

A change intended to address an identified problem or cause.

IN USE assign a corrective action

action owner

The named person responsible for carrying out an agreed action.

IN USE name the action owner

effectiveness measure

A way to check whether an action achieved its intended result.

IN USE define an effectiveness measure

balancing measure

A check for unintended effects of a change.

IN USE include a balancing measure

learning feedback

Information returned to staff about findings and improvements.

IN USE provide learning feedback

Precision check

An incident report and a clinical record serve different purposes; follow local requirements for both. A completed training session shows delivery, not necessarily reduced risk. Match follow-up measures to the actual finding and monitor unintended effects.

Phrases you can actually use

C Notice the language / 1 of 2

STATE THE EVENT

"A fall was recorded at 11:05 a.m."

LIMIT FIRSTHAND KNOWLEDGE

"I did not witness the fall."

NAME THE UNKNOWN

"The call-bell position at the time has not been verified."

RESPECT THE CLINICAL RESPONSE

"The safety review does not replace immediate patient assessment and care."

ATTRIBUTE THE ACCOUNT

"The patient later reported standing to reach water."

CHALLENGE THE INFERENCE

"The draft does not establish that the colleague was careless."

PRESERVE THE SEQUENCE

"Separate what happened before, during, and after the event."

SEEK RELEVANT EVIDENCE

"Which records or accounts can clarify the conditions at that time?"

08

Caused by

Requires more support than a sequence or an assumption.

Patient reported

Attributes information without claiming you witnessed it.

No blame

A fair review is not a promise that conduct will never be examined.

Phrases you can actually use

C Notice the language / 2 of 2

AVOID HINDSIGHT

"Evaluate what was known then, not only what became clear afterward."

MATCH THE ACTION

"Which identified factor would this proposed change address?"

DISTINGUISH DELIVERY

"Attendance confirms the briefing occurred, not that risk decreased."

WATCH FOR TRADE-OFFS

"Include a check for unintended delays or workarounds."

KEEP ACCOUNTABILITY FAIR

"Review actions and system conditions without prejudging the outcome."

NAME AN OWNER

"Assign the agreed action to a specific responsible person."

CHECK EFFECTIVENESS

"Measure whether the intended condition actually improved."

RETURN THE LEARNING

"Share the supported findings and next steps through the appropriate process."

08

Root cause

Avoid using the phrase as a shortcut for the first explanation suggested.

Completed training

Describes an activity, not automatically its practical effect.

Resolved

Use only when the defined problem and follow-up criteria have actually been addressed.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which opening gives the strongest factual foundation?

- A** The call bell was definitely unreachable because a fall occurred.
- B** Every system failed at exactly the same time.
- C** A fall was recorded at 11:05; I did not witness it; the patient later reported standing to reach water.
- D** A careless colleague caused the fall.

2. Which statement needs verification before being presented as fact?

- A** The patient later reported standing for water.
- B** A fall is recorded at eleven oh five.
- C** The call bell was out of reach at the time.
- D** Beth did not witness the fall.

3. A later review identifies inconsistent call-bell positioning. Which measure tests a related improvement?

- A** The number of slides in a staff presentation
- B** Whether a training invitation was sent
- C** The number of times a reminder email was opened
- D** A defined audit of whether call bells are appropriately accessible in the relevant setting

4. What does a just-culture approach require here?

- A** Assume everyone is careless after an adverse outcome.
- B** Promise that no conduct will ever be reviewed.
- C** Remove patient accounts because staff did not witness the event.
- D** Examine supported actions and system conditions with fair accountability.

Answers and explanations: page 90.

Report the fall without inventing its cause

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

hindsight bias / effectiveness measure / evidence preservation / action owner / learning feedback / firsthand observation / just culture / Call-bell access / causal assumption / event chronology

08

- 01 **Beth:** I am reviewing the safety report for the fall at eleven oh five. The immediate clinical response followed our process, but the draft explanation goes beyond what I know.
- 02 **Arun:** Let us start with the ¹ _____. What is recorded, what did you observe yourself, and what was reported afterward by somebody else?
- 03 **Beth:** I did not witness the fall. The patient later said they had stood to reach water. I can attribute that account, but I cannot describe the fall as an eyewitness.
- 04 **Arun:** Make the limit on ² _____ explicit. The patient's account remains relevant; it should not be erased simply because you did not see the event yourself.
- 05 **Beth:** The draft says a colleague was careless. I cannot identify evidence supporting that description, and it might steer the next reviewer before the circumstances are reconstructed.
- 06 **Arun:** That is a ³ _____, not an established finding. Remove the unsupported conclusion through the appropriate correction process while retaining the actual event and relevant accounts.
- 07 **Beth:** It also says the call bell was out of reach. We need to establish its position at the time rather than infer it from the fact that the patient stood.
- 08 **Arun:** Yes. ⁴ _____ is a relevant question, but not yet a verified explanation. Identify which information could clarify that condition and preserve the distinction in the report.
- 09 **Beth:** We may also need the surrounding care record and staff accounts. They could help reconstruct the sequence, although no single source should be treated as complete automatically.
- 10 **Arun:** Follow the appropriate ⁵ _____ process. Keep records and accounts accurate and traceable rather than rewriting them to fit the first theory suggested at the meeting.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 73.

- 11 **Beth:** Some staff fear that questioning the draft removes accountability. Others think reviewing individual actions means automatically blaming someone.
- 12 **Arun:** A ⁶ _____ avoids both shortcuts. We examine actions and system conditions fairly; we do not decide guilt from the outcome or promise that conduct can never be reviewed.
- 13 **Beth:** Knowing the outcome makes it tempting to say the next step should have been obvious. But we need to understand what information people actually had at the time.
- 14 **Arun:** That helps avoid ⁷ _____. Reconstruct the working situation before judging decisions. The review should explain what is supported, what remains uncertain, and what further evidence is needed.
- 15 **Beth:** Suppose a later review finds inconsistent positioning of the call bell. A reminder session alone would not tell us whether the intended bedside condition actually improved.
- 16 **Arun:** We would need an ⁸ _____ matched to that finding, such as a defined accessibility audit in the relevant setting. Attendance would show delivery, not the practical result.
- 17 **Beth:** We should assign the change to someone and state when follow-up will be checked. Otherwise we have only a general promise.
- 18 **Arun:** Name the ⁹ _____ and the review point. Include an appropriate check for unintended effects rather than assume every change is beneficial in every respect.
- 19 **Beth:** I will revise the report to separate the recorded fall, the patient's account, and the unanswered questions. Any later conclusion should identify the evidence supporting it.
- 20 **Arun:** Then provide ¹⁰ _____ through the appropriate route. Staff need the supported findings and next actions, not a repeated accusation or a claim that the event is explained before review.

08

ANSWERS / EXPLANATIONS: PAGE 91

1. event chronology | 2. firsthand observation | 3. causal assumption | 4. Call-bell access | 5. evidence preservation | 6. just culture | 7. hindsight bias | 8. effectiveness measure | 9. action owner | 10. learning feedback

Delivery is not effectiveness

F Say it

After an identified equipment-location problem, a team gives a briefing to 20 staff. No check of equipment accessibility has yet occurred. A review asks whether the practical condition improved.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

activity / unchecked / effectiveness / delivered

08

Lead: "The briefing was ¹ _____ to twenty staff."

Reviewer: "That is evidence of the ² _____, not the practical result."

Lead: "Equipment accessibility remains ³ _____."

Reviewer: "Use a relevant measure of ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 91

1. delivered | 2. activity | 3. unchecked | 4. effectiveness

Rehearse with a fixed script

1. Check your answers, then read turns 1-10 with a partner using the corrected words.
2. Switch roles for turns 11-20. Stress words that distinguish facts from conditions or estimates.
3. Complete and check the four-line exchange. Read it twice, switching roles and keeping the printed facts.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

01 Briefing and language checks

Shift Handoffs and Clinical Prioritization

A Briefing checks / page 4

1. What is known about the requested result?

C. It is pending, with no value available to Maya.

The briefing states that the result is pending and no value has been seen.

2. Which responsibility is transferring?

D. Checking for the result and contacting the responsible clinician when available

Jonas is taking over the specified follow-up, not a new clinical decision.

3. What completes the verbal transfer?

C. Jonas confirms the task and next action, and Maya checks that confirmation.

Mutual confirmation establishes what the receiving nurse understood and accepted.

D Language checks / page 9

1. Which opening preserves the known facts?

B. At seven, the result is still pending; Jonas will follow it and contact the responsible clinician.

The first statement preserves pending status and names the agreed follow-up responsibility.

2. Jonas says, "I will review the normal result." What needs correction?

B. No value is available, so normal is unsupported.

No result value has been received, so neither normal nor abnormal is established.

3. Which response is a useful check-back?

B. I will check for the result and contact the responsible clinician when it is available.

The response repeats both the task and the action requested when the result arrives.

4. What should happen if an urgent change occurs before the result?

A. Use the appropriate clinical escalation process without waiting for that result.

Pending information does not justify delaying the appropriate response to an urgent clinical change.

01 Conversation answers

Shift Handoffs and Clinical Prioritization

E Dialogue gaps

Pages 10-11

1. current status. Turn 2. Current status identifies what is actually known at the stated handoff time.

2. pending result. Turn 4. Pending result means no available result has yet supplied the value being discussed.

3. follow-up owner. Turn 6. The follow-up owner is the named person accepting responsibility for the outstanding action.

4. care plan. Turn 8. The care plan remains the documented approach unless an appropriately authorized change is made.

5. source record. Turn 10. The source record provides the original information against which summaries should be checked.

6. escalation route. Turn 12. The escalation route provides the appropriate path for further clinical attention when needed.

7. check-back. Turn 14. A check-back repeats the received task so the sender can verify accurate understanding.

8. outstanding task. Turn 16. An outstanding task remains open until the required follow-up has actually occurred.

9. responsibility transfer. Turn 18. Responsibility transfer records the accepted movement of a defined duty between professionals.

10. continuity of care. Turn 20. Continuity of care connects the unfinished work across the shift boundary with an explicit owner.

F Transfer conversation

Page 12

1. pending

Therapist: "The report is available, but clinical review is pending."

Availability of a report does not mean the responsible clinician has reviewed it.

2. action

Nurse: "Who owns the next action?"

The exchange needs an explicit owner for the remaining contact task.

3. contact

Therapist: "I will contact the responsible clinician."

The supplied agreement assigns that contact to the incoming therapist.

4. interpretation

Nurse: "Do not add an unsupported interpretation."

No clinical interpretation appears in the supplied facts for this exercise.

02 Briefing and language checks

Patient Assessment and Escalation

A Briefing checks / page 13

1. What changed between the supplied observations?

C. Alex now stops mid-sentence to catch a breath.

The brief contrasts the earlier observation with the new interruption of speech.

2. What is the source of "I feel worse than earlier"?

D. Alex's own report

The statement is attributed directly to the patient, rather than to a clinician or test.

3. What is Priya already doing?

B. Using the local urgent-response process

The briefing explicitly places the communication within an immediate local response.

D Language checks / page 18

1. Which opening is most useful during the response?

B. At 10:20 Alex is stopping mid-sentence to catch a breath, unlike at ten; I need urgent review.

The message gives the observed change, comparison, time, and requested response without diagnosing.

2. Which statement properly separates sources?

D. Alex reports feeling worse; I observe pauses to catch a breath.

The first statement distinguishes the patient's account from the nurse's observation.

3. Dan has not acknowledged the request. Which response fits the brief?

A. Continue the appropriate urgent escalation process rather than assume silence means acceptance.

A sent request is not confirmed receipt, and the urgent concern remains actionable.

4. What should the later record describe?

C. Actual observations, attributed reports, relevant times, and the response taken

A factual escalation record preserves both the clinical information and what actually occurred.

02 Conversation answers

Patient Assessment and Escalation

E Dialogue gaps

Pages 19-20

1. urgent review. Turn 2. Urgent review is the prompt clinical assessment Priya is requesting in response to the change.

2. patient-reported symptom. Turn 4. The patient's description is a reported experience, distinct from the nurse's direct observation.

3. change from baseline. Turn 6. The comparison shows a difference from the earlier reference observation, not an unchanged status.

4. clinical concern. Turn 8. The clinical concern is the stated reason for prompt professional attention to the observed change.

5. situational awareness. Turn 10. Situational awareness keeps the team aligned on the current condition, relevant change, and response.

6. closed-loop communication. Turn 12. Closed-loop communication includes confirmation of receipt and meaning rather than reliance on sending alone.

7. time course. Turn 14. The time course describes how the concern developed across the supplied observations.

8. clinical uncertainty. Turn 16. Clinical uncertainty identifies what remains unknown without reducing the need for an appropriate response.

9. escalation record. Turn 18. The escalation record documents the concern, communication, timing, and actual response for continuity.

10. reassessment. Turn 20. Reassessment is a further clinical assessment whose findings must reflect what is actually established.

F Transfer conversation

Page 21

1. earlier

Therapist: "The noon entry is an earlier observation."

Noon precedes the new finding at two and cannot establish the current status.

2. change

Colleague: "The two o'clock finding describes a change."

The supplied facts explicitly identify something new rather than an unchanged condition.

3. underway

Therapist: "The appropriate response is already underway."

The briefing says the therapist has used the appropriate response route.

4. assessment

Colleague: "We must not substitute the old note for a current assessment."

An earlier record does not replace assessment of the newly identified change.

03 Briefing and language checks

Medication Safety and Allergy Clarification

03

A Briefing checks / page 22

1. Which conflict needs review?

B. The record says no known drug allergies, while Rosa reports a previous rash after an antibiotic.

The discrepancy is between the documented status and an incompletely identified historical patient report.

2. Which details remain unknown?

D. The antibiotic name and exact date

The briefing explicitly identifies the missing drug name and date.

3. What can this language exercise establish?

C. How to communicate the discrepancy accurately for appropriate review

The supplied facts support accurate communication, not a clinical conclusion or medication decision.

D Language checks / page 27

1. Which message best supports review?

C. The record says no known drug allergies; Rosa reports a past rash after an unnamed antibiotic.

The accurate message preserves both sources and the limits of the reported history.

2. Which inference is unsupported?

B. The rash was definitely caused by an immune-mediated drug allergy.

Timing and a patient report alone do not establish the reaction's mechanism or cause.

3. What should happen to the unresolved information?

D. Keep it visible and route it for appropriate clinical clarification.

Incomplete information still matters and must not be silently removed or prematurely closed.

4. Which question seeks a useful missing detail without suggesting the answer?

B. Do you remember the medicine's name or have a prior record we can appropriately check?

The neutral question seeks identifying evidence without supplying an unsupported answer.

03 Conversation answers

Medication Safety and Allergy Clarification

03

E Dialogue gaps

Pages 28-29

- 1. discrepancy.** Turn 2. The discrepancy is the difference between the recorded status and the patient's historical report.
- 2. patient account.** Turn 4. The patient account attributes the historical information to Rosa without making it a confirmed diagnosis.
- 3. allergy status.** Turn 6. Allergy status is the documented position that requires clarification when new information conflicts with it.
- 4. temporal association.** Turn 8. Temporal association preserves the reported timing without claiming that causation has been established.
- 5. corroborating record.** Turn 10. A corroborating record may help identify the medicine or clarify the previous reaction history.
- 6. generic name.** Turn 12. The generic name identifies the medicine's active substance rather than relying only on a remembered product brand.
- 7. Unable to ascertain.** Turn 14. Unable to ascertain describes information that cannot currently be established, not an absence of allergy history.
- 8. pharmacist review.** Turn 16. Pharmacist review is the professional assessment of the medication-related discrepancy, not a clerical status change.
- 9. prescriber clarification.** Turn 18. Prescriber clarification obtains the authorized prescribing professional's response to the medication question.
- 10. Record reconciliation.** Turn 20. Record reconciliation addresses the documented difference through an authorized process rather than deleting one source.

F Transfer conversation

Page 30

1. unknown

Nurse: "The medicine name is unknown."

The patient's inability to name the medicine leaves that detail unestablished.

2. absent

Colleague: "That does not prove the history is absent."

Missing identification does not establish that no reaction history exists.

3. report

Nurse: "Preserve the patient's report."

The supplied history should remain attributed and visible for appropriate review.

4. clarification

Colleague: "Route the discrepancy for qualified clarification."

The unresolved difference requires appropriate review, not an unsupported negative entry.



04 Briefing and language checks

Patient Education and Teach-Back

04

A Briefing checks / page 31

1. Which appointment details are supplied?

A. Thursday, 10:30 a.m., reception B

The approved sheet gives Thursday at ten thirty in the morning at reception B.

2. What does Victor's Friday answer reveal?

D. A misunderstanding that needs clarification

The supplied facts show a mismatch in understanding, not refusal or a changed booking.

3. What is the printed clinic number for in this case?

B. Appointment questions

The brief explicitly limits this number's stated purpose to appointment questions.

D Language checks / page 36

1. Which question checks the supplied information most effectively?

A. In your own words, when and where will you return, and what will you bring?

The focused question requires the patient to explain the actual plan rather than simply agree.

2. Victor says Friday. Which response is best?

C. I may not have made the day clear; the sheet says Thursday. Let us check that part again.

The response corrects the supplied detail without blame and includes another check.

3. Which entry is supported after Victor accurately restates the corrected details?

C. Appointment day, time, reception, and medication-list instruction checked through teach-back

The completed check supports the specific details verified, not unlimited claims about future behavior.

4. Victor asks whether the clinic number replaces urgent care routes. What preserves the brief?

C. No; its stated purpose here is appointment questions, and urgent concerns use the appropriate clinical route.

The supplied number has a limited appointment purpose and does not replace appropriate urgent attention.

04 Conversation answers

Patient Education and Teach-Back

E Dialogue gaps

Pages 37-38

1. appointment. Turn 2. Appointment identifies the specific later visit being discussed, not the entire discharge plan.

2. medication list. Turn 4. The medication list is the supplied document Victor should bring to the visit.

3. detail. Turn 6. Detail refers to the particular day that Victor confused with another appointment.

4. reception B. Turn 8. Reception B is the location explicitly stated in the approved appointment instructions.

5. current. Turn 10. Current distinguishes the up-to-date medication list from an older version that may no longer be accurate.

6. contact. Turn 12. Contact refers to the printed route for appointment questions, whose purpose must remain clear.

7. appointment questions. Turn 14. Appointment questions is the limited purpose assigned to the printed clinic number in this scenario.

8. location. Turn 16. Location identifies Victor's remaining practical question about where to attend.

9. access barrier. Turn 18. An access barrier is a practical difficulty, here uncertainty about finding the right reception.

10. plan. Turn 20. Plan summarizes the corrected appointment facts that Victor has now checked with Leila.

F Transfer conversation

Page 39

1. location

Nurse: "The day and time match; let us clarify the location."

The only mismatch in the supplied response concerns the reception.

2. C

Patient: "The note says reception C."

The approved appointment note explicitly names reception C.

3. words

Nurse: "Please explain that corrected detail in your own words."

Teach-back checks the person's understanding rather than asking only for agreement.

4. afternoon

Patient: "I will use reception C on Tuesday at two in the afternoon."

Two p.m. means two in the afternoon, preserving the supplied appointment time.

04

05 Briefing and language checks

Interprofessional Rounds

A Briefing checks / page 40

1. What was actually assessed?

D. A 40-meter corridor walk with a wheeled walker and supervision

The supplied assessment concerns a corridor walk under stated equipment and supervision conditions.

2. What remains unresolved?

D. Stair access to the stated home setting

The home has twelve steps, but the relevant stair task has not been assessed.

3. What is the team's immediate communication need?

B. A precise finding and a clear unresolved question for the appropriate review

The case calls for accurate shared information before the qualified discharge decision.

D Language checks / page 45

1. Which summary is most accurate?

C. Walked 40 meters with a wheeled walker and supervision; stairs not assessed.

The accurate statement preserves the assessed task and its conditions without inventing stair findings.

2. Why does the home information matter?

C. The destination requires a task that the corridor assessment did not establish.

The home requires stair access, which remains distinct from the assessed corridor walk.

3. How should Omar respond to pressure for a noon summary?

B. Report the specific assessment and unresolved stair question for the qualified team.

Accurate reporting allows the appropriate team to address the actual gap despite scheduling pressure.

4. Which statement incorrectly treats missing evidence as a negative finding?

D. Eva established that the patient cannot use stairs.

The supplied facts state no stair assessment, not a demonstrated inability to use stairs.

05 Conversation answers

Interprofessional Rounds

E Dialogue gaps

Pages 46-47

1. functional assessment. Turn 2. Functional assessment concerns the specific task and conditions actually evaluated, not a general clearance.

2. supervision. Turn 4. Supervision was required during the observed walk, so independent mobility would overstate the finding.

3. stair negotiation. Turn 6. Stair negotiation is the unassessed task required by the stated access to the home.

4. unassessed task. Turn 8. An unassessed task lacks an evaluation; that is different from a demonstrated inability.

5. home environment. Turn 10. The home environment supplies practical conditions that the discharge plan must address.

6. equipment provision. Turn 12. Equipment provision concerns actual arrangements for equipment, not merely its use during an assessment.

7. discharge readiness. Turn 14. Discharge readiness concerns the relevant conditions for leaving, not the proposed departure time alone.

8. shared goal. Turn 16. A shared goal allows the team to coordinate a workable plan without concealing unresolved evidence.

9. discharge destination. Turn 18. The discharge destination identifies the actual setting whose access and support requirements need consideration.

10. team decision. Turn 20. The team decision must reflect the appropriate review rather than an inference from a vague mobility summary.

F Transfer conversation

Page 48

05

1. indoors

Therapist: "The observed walk was indoors."

The supplied assessment took place indoors, not on outdoor ground.

2. assistance

Nurse: "It required assistance."

The briefing explicitly includes assistance as a condition of the observed walk.

3. unassessed

Therapist: "Uneven outdoor ground is unassessed."

No assessment of that separate task is supplied in this case.

4. independence

Nurse: "The summary must not claim outdoor independence."

An assisted indoor walk does not establish independent outdoor performance.

06 Briefing and language checks

Documentation and Charting

A Briefing checks / page 49

1. What did the patient actually request?

B. Rest before washing

The supplied quotation concerns the timing of washing, not every aspect of care.

2. Which times need to be distinguished?

C. Event at 2:20 p.m. and entry at 2:45 p.m.

The briefing gives separate times for the interaction and documentation.

3. What was agreed?

D. Elena would return at 3 p.m.

The agreement is a future return time, not a completed care event.

D Language checks / page 54

1. Which replacement note best matches the event?

A. At 2:20, patient said, "I want to rest before washing." Return at 3 offered and agreed.

The replacement preserves the quotation, timing, offer, and agreement without adding unsupported judgments.

2. Which claim is not supported?

B. The patient refused every type of care.

The brief explicitly says no other care was discussed, so the universal refusal claim is unjustified.

3. How should the two times be handled?

D. Identify the event time and preserve the actual entry time under local procedure.

The two times describe different events and should not be conflated or concealed.

4. At 2:45, which follow-up statement is accurate?

B. Return at 3 p.m. agreed; outcome not yet recorded.

The future return has been agreed but its actual outcome is not yet known.

06 Conversation answers

Documentation and Charting

E Dialogue gaps

Pages 55-56

- 1. behavioral description.** Turn 2. A behavioral description identifies what was said or done instead of relying on a judgmental label.
- 2. direct quotation.** Turn 4. The direct quotation preserves the patient's actual words and the limited request they express.
- 3. template text.** Turn 6. Template text is preset wording that must still match the facts of the individual interaction.
- 4. event time.** Turn 8. Event time identifies when the patient interaction occurred, distinct from when the note was entered.
- 5. agreed follow-up.** Turn 10. Agreed follow-up is the accepted future plan, not evidence that the later visit has occurred.
- 6. clinical inference.** Turn 12. A clinical inference must be distinguished from observed facts and cannot be invented to explain the event.
- 7. amendment.** Turn 14. An amendment corrects a record through the approved process without concealing the original history.
- 8. audit trail.** Turn 16. The audit trail provides traceability of entries and changes, supporting reliable correction rather than concealment.
- 9. documentation scope.** Turn 18. Documentation scope limits the entry to what the supplied evidence and relevant professional process actually support.
- 10. Objective documentation.** Turn 20. Objective documentation records supported facts and attributed information without the unsupported label in the original draft.

F Transfer conversation

Page 57

1. future

Reviewer: "Noon is still in the future."

At eleven, the agreed noon visit has not yet occurred.

2. planned

Author: "The visit is planned, not completed."

The supplied fact is an agreement to return, not a completed education session.

3. process

Reviewer: "Correct the statement through the approved process."

An inaccurate entry should be corrected transparently under the applicable documentation procedure.

4. outcome

Author: "The later entry must reflect the actual outcome."

The result of the visit can be documented only after it is known.

07 Briefing and language checks

Difficult Families and Boundaries

A Briefing checks / page 58

1. What permission is supplied?

D. The patient has agreed to include Anika in relevant updates.

The permission concerns relevant updates to Anika, not unlimited authority or disclosure.

2. What can Luis promise?

A. His own status update by 4 p.m.

The brief gives Luis control of his update, not the clinician's schedule or clinical conclusion.

3. How is Anika's behavior described?

B. Frustrated, without threats

The supplied facts identify frustration but explicitly state that no threats are being made.

D Language checks / page 63

1. Which opening best acknowledges the concern?

D. You have been waiting for a clear explanation; let me check the unanswered question and arrange the right contact.

The opening recognizes the missing explanation and moves toward an appropriate response without unsupported reassurance.

2. Anika asks, "Will the clinician be here at four?" What is accurate?

A. Four is my promised status update; a clinician meeting time is not yet confirmed.

The brief distinguishes Luis's own update from an unconfirmed clinician meeting.

3. What should Luis avoid inferring from permission to share updates?

D. That Anika automatically has full decision-making authority

Being included in communication does not itself confer authority to make every decision.

4. At four, no meeting time is confirmed. What honors the commitment?

A. Give Anika the current status and a clear next communication step.

Luis promised an update, so honest communication remains due even while the meeting is pending.

07 Conversation answers

Difficult Families and Boundaries

E Dialogue gaps

Pages 64-65

- 1. clear explanation.** Turn 2. Clear explanation identifies the unmet communication need Luis is acknowledging rather than dismissing.
- 2. updates.** Turn 4. Updates are the relevant communications the patient has permitted Anika to receive in this case.
- 3. responsible clinician.** Turn 6. The responsible clinician is the appropriate contact for the clinical explanation sought in this scenario.
- 4. status update.** Turn 8. A status update reports what has been established and what remains pending, not necessarily a completed clinical explanation.
- 5. update time.** Turn 10. Update time identifies Luis's own communication commitment, not a clinician meeting that has not been arranged.
- 6. reassurance.** Turn 12. Reassurance about a normal result would lack a clinical basis in the facts supplied here.
- 7. concern.** Turn 14. Concern refers to Anika's complaint about the communication experience, which can be passed on accurately.
- 8. decision-making authority.** Turn 16. Decision-making authority is distinct from permission to receive relevant information and requires the applicable basis.
- 9. named contact.** Turn 18. The named contact is the identified person accountable for this promised follow-up communication.
- 10. commitment.** Turn 20. Commitment refers to the promised update Luis can control even if the meeting remains unconfirmed.

F Transfer conversation

Page 66

1. time

Family member: "Is five the conference time?"

The question asks whether the update time also establishes a conference appointment.

2. update

Therapist: "Five is my update; the conference is not yet booked."

The only confirmed promise concerns the therapist's communication.

3. status

Family member: "Please explain the current status even if it is pending."

The family requests accurate information about arrangements, not an invented booking.

4. commitment

Therapist: "I will keep my communication commitment."

The update can still be delivered even when another person's meeting is unconfirmed.

08 Briefing and language checks

Safety Events and Just Culture

A Briefing checks / page 67

1. Which event is supplied as recorded?

C. A fall at 11:05 a.m.

The briefing establishes the recorded fall and time, not the draft's causal claims.

2. What is the source of the water detail?

A. The patient's later report

The patient reported standing to reach water; Beth did not witness the event.

3. What is the purpose of this review?

D. Reconstruct supported facts and unresolved details before conclusions

The review follows the clinical response and seeks evidence before causal conclusions.

D Language checks / page 72

1. Which opening gives the strongest factual foundation?

C. A fall was recorded at 11:05; I did not witness it; the patient later reported standing to reach water.

The factual opening identifies the event, the limit of firsthand knowledge, and the source of the account.

2. Which statement needs verification before being presented as fact?

C. The call bell was out of reach at the time.

The briefing explicitly says the call-bell position has not been established.

3. A later review identifies inconsistent call-bell positioning. Which measure tests a related improvement?

D. A defined audit of whether call bells are appropriately accessible in the relevant setting

A relevant accessibility audit measures the intended condition, whereas delivery counts do not.

4. What does a just-culture approach require here?

D. Examine supported actions and system conditions with fair accountability.

Fair learning combines evidence about systems and behavior without automatic blame or automatic absolution.

08 Conversation answers

Safety Events and Just Culture

E Dialogue gaps

Pages 73-74

- 1. event chronology.** Turn 2. Event chronology reconstructs the supported sequence and times before assigning a cause.
- 2. firsthand observation.** Turn 4. Firsthand observation concerns what the reporter directly witnessed, which Beth explicitly did not do here.
- 3. causal assumption.** Turn 6. A causal assumption presents an explanation as established before the supporting evidence is available.
- 4. Call-bell access.** Turn 8. Call-bell access is a specific environmental condition requiring verification, not an inference from the fall alone.
- 5. evidence preservation.** Turn 10. Evidence preservation protects relevant information so the review can reconstruct events accurately.
- 6. just culture.** Turn 12. Just culture combines fair examination of individual accountability with learning about system conditions.
- 7. hindsight bias.** Turn 14. Hindsight bias makes an earlier decision seem obvious after the outcome is already known.
- 8. effectiveness measure.** Turn 16. An effectiveness measure checks whether the intended practical condition improved rather than merely counting an activity.
- 9. action owner.** Turn 18. The action owner is accountable for carrying out the agreed improvement and its follow-up.
- 10. learning feedback.** Turn 20. Learning feedback returns supported findings and practical next steps to the people involved in improving care.

F Transfer conversation

Page 75

1. delivered

Lead: "The briefing was delivered to twenty staff."

Attendance supports that the briefing took place for the stated group.

2. activity

Reviewer: "That is evidence of the activity, not the practical result."

Delivery describes what the team did, not whether the intended condition changed.

3. unchecked

Lead: "Equipment accessibility remains unchecked."

The supplied facts say no accessibility check has yet occurred.

4. effectiveness

Reviewer: "Use a relevant measure of effectiveness."

The review needs evidence that the action improved the intended practical condition.

08

Find the move you need

Use these as precise language models. Replace details only when you know the actual facts, authority, and agreed conditions.

01 Shift Handoffs and Clinical Prioritization

Open the transfer: I am handing over one result that is still pending.

Separate known from unknown: I have not seen a result value yet.

Check understanding: Please repeat the outstanding action so we can confirm it.

Full phrase set: pages 7-8 | Dialogue: pages 10-11

02 Patient Assessment and Escalation

State the change: This is different from the observation at ten o'clock.

Request review: I need an urgent review now.

Keep the concern clear: I am concerned about the change, not just the wording in the earlier note.

Full phrase set: pages 16-17 | Dialogue: pages 19-20

03 Medication Safety and Allergy Clarification

State the discrepancy: The allergy field and the patient's account do not match.

Avoid diagnosis: The cause of that historical reaction is not established.

Avoid false certainty: Unknown is not the same as no known allergies.

Full phrase set: pages 25-26 | Dialogue: pages 28-29

04 Patient Education and Teach-Back

Invite the plan: In your own words, when and where will you return?

Point to the source: Let us look at the appointment line together.

Confirm what to bring: Please bring the current medication list shown on the sheet.

Full phrase set: pages 34-35 | Dialogue: pages 37-38

Keep the conversation productive

Use these as precise language models. Replace details only when you know the actual facts, authority, and agreed conditions.

05 Interprofessional Rounds

State the finding: Eva observed a 40-meter corridor walk.

Clarify the home: The stated home access has twelve steps and no elevator.

Preserve professional scope: The relevant qualified team must review the discharge implications.

Full phrase set: pages 43-44 | Dialogue: pages 46-47

06 Documentation and Charting

Replace the label: Describe what was said and what you observed.

Record the offer: I offered to return at 3 p.m.

Keep plans prospective: A planned return is not a completed visit.

Full phrase set: pages 52-53 | Dialogue: pages 55-56

07 Difficult Families and Boundaries

Acknowledge the experience: You have been waiting for a clear explanation, and that is frustrating.

Keep the role clear: I can explain the next step, but I cannot invent an interpretation of the result.

Avoid unsupported reassurance: I cannot tell you the result is normal from the information we have here.

Full phrase set: pages 61-62 | Dialogue: pages 64-65

08 Safety Events and Just Culture

State the event: A fall was recorded at 11:05 a.m.

Challenge the inference: The draft does not establish that the colleague was careless.

Seek relevant evidence: Which records or accounts can clarify the conditions at that time?

Full phrase set: pages 70-71 | Dialogue: pages 73-74

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

access barrier	identify an access barrier	33
access requirement	clarify access requirements	42
acknowledgment	obtain an acknowledgment	6
action list	review the action list	6
action owner	name the action owner	69
action step	identify the next action step	33
active listening	demonstrate active listening	59
activity tolerance	describe activity tolerance	41
addendum	make an authorized addendum	50
adverse drug reaction	document an adverse drug reaction	23
agreed follow-up	record agreed follow-up	51
allergy alert	review an allergy alert	24
allergy status	verify allergy status	23
ambulation	describe observed ambulation	41
amendment	request an amendment	50
appointment window	clarify the appointment window	32
approved abbreviation	use approved abbreviations	51
assertive statement	make an assertive statement	15
assessment finding	report assessment findings	14
audit trail	preserve the audit trail	50
author attribution	maintain author attribution	51
authorized recipient	confirm the authorized recipient	59
autopopulated field	verify autopopulated fields	51
balancing measure	include a balancing measure	69

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

baseline compare with the baseline	5
bedside review arrange a bedside review	15
behavioral description provide a behavioral description	50
brand name record the brand name	23
call-bell access check call-bell access	69
call-out make a focused call-out	15
capacity assessment document a capacity assessment accurately	51
care declined specify the care declined	51
care plan continue the care plan	6
caregiver involvement confirm caregiver involvement	32
causal assumption challenge a causal assumption	68
chain of command use the chain of command	15
change from baseline report a change from baseline	14
check-back use a check-back	5
chunk and check use chunk and check	32
clarifying question ask a clarifying question	59
clinical concern state the clinical concern	14
clinical explanation arrange a clinical explanation	60
clinical inference separate clinical inference from observation	50
clinical observation communicate a clinical observation	14
clinical priority communicate clinical priority	6
clinical rationale explain the clinical rationale	15
clinical record maintain the clinical record	69
clinical uncertainty communicate clinical uncertainty	14

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

closed-loop communication	use closed-loop communication	15
communication preference	record communication preferences	60
complaint pathway	explain the complaint pathway	60
comprehension check	make a comprehension check	33
confidentiality	maintain confidentiality	59
consent discussion	arrange a consent discussion	60
contact pathway	clarify the contact pathway	33
contemporaneous note	make a contemporaneous note	51
contingency	clarify the contingency	5
continuity of care	support continuity of care	6
contraindication	check a contraindication	24
contributing factor	investigate contributing factors	68
copy-forward	verify copy-forward content	51
corrective action	assign a corrective action	69
corroborating record	seek a corroborating record	24
current status	summarize current status	5
de-escalation	use de-escalation skills	59
demonstration	provide a demonstration	33
deterioration	recognize possible deterioration	14
differential diagnosis	discuss a differential diagnosis	14
direct quotation	include a relevant direct quotation	50
discharge barrier	communicate a discharge barrier	42
discharge destination	confirm the discharge destination	42
discharge instructions	review discharge instructions	32

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

discharge readiness	review discharge readiness	42
discipline-specific finding	share discipline-specific findings	42
discrepancy	escalate a discrepancy	24
documentation scope	keep within documentation scope	51
drug allergy	assess a suspected drug allergy	23
education record	update the education record	33
effectiveness measure	define an effectiveness measure	69
empathic acknowledgment	offer empathic acknowledgment	59
entry time	preserve the entry time	50
environmental condition	verify environmental conditions	69
equipment provision	verify equipment provision	42
escalation record	complete the escalation record	15
escalation route	confirm the escalation route	6
event chronology	reconstruct the event chronology	68
event time	record the event time	50
evidence preservation	support evidence preservation	69
expectation management	improve expectation management	60
factual correction	document a factual correction	51
family liaison	coordinate family liaison	59
family meeting	request a family meeting	60
firsthand observation	distinguish firsthand observation	68
follow-up appointment	confirm the follow-up appointment	32
follow-up owner	name the follow-up owner	5
formulation	confirm the formulation	23

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

functional assessment report a functional assessment	41
functional limitation describe a functional limitation	42
gait assess gait	41
generic name verify the generic name	23
handoff give a structured handoff	5
health literacy support health literacy	32
hindsight bias avoid hindsight bias	68
home environment assess the home environment	42
human factors apply human-factors analysis	68
hypersensitivity describe suspected hypersensitivity	23
I-PASS use an I-PASS handoff	6
immediate response document the immediate response	69
incident report complete an incident report	69
independent mobility verify independent mobility	41
informed refusal distinguish an informed refusal	51
interprofessional rounds contribute to interprofessional rounds	41
intolerance clarify a reported intolerance	23
just culture support a just culture	68
language access arrange language access	32
late entry identify a late entry	50
learning feedback provide learning feedback	69
medication administration record check the medication administration record	24
medication list bring a current medication list	32
medication reconciliation complete medication reconciliation	24

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

misunderstanding clarify a misunderstanding	33
mobility aid specify the mobility aid	41
monitoring plan clarify the monitoring plan	15
named contact provide a named contact	60
no known drug allergies verify a no-known-drug-allergies entry	24
nonjudgmental wording use nonjudgmental wording	33
objective documentation use objective documentation	50
onset clarify symptom onset	14
outcome bias recognize outcome bias	68
outstanding task identify outstanding tasks	5
patient account preserve the patient account	24
patient identifier verify patient identifiers	5
patient preference respect patient preferences	59
patient safety event report a patient safety event	68
patient-reported symptom record a patient-reported symptom	14
pending result follow a pending result	5
permission to share verify permission to share	59
pharmacist review obtain pharmacist review	24
physical assistance specify physical assistance	41
plain language explain in plain language	32
prescriber clarification request prescriber clarification	24
professional boundary maintain a professional boundary	59
psychological safety strengthen psychological safety	68
qualified interpreter request a qualified interpreter	32

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

reaction history obtain a reaction history	23
read-back complete a read-back	6
reassessment communicate reassessment findings	15
receiving clinician confirm the receiving clinician	5
record integrity protect record integrity	50
record reconciliation track record reconciliation	24
relevant history provide relevant history	5
rephrasing try a clearer rephrasing	33
response activation confirm response activation	14
response time record the response time	15
responsibility transfer document responsibility transfer	6
return demonstration observe a return demonstration	33
root cause analysis contribute to root cause analysis	69
route of administration verify the route of administration	23
safety boundary apply an appropriate safety boundary	60
safety concern raise a safety concern	15
SBAR structure the report with SBAR	6
scope of practice work within scope of practice	59
secondhand account attribute a secondhand account	68
self-management support self-management	32
shared decision-making support shared decision-making	60
shared goal establish a shared goal	42
shift overlap use the shift overlap	5
situational awareness maintain situational awareness	15

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

source record check the source record	6
stair negotiation assess stair negotiation	41
status update provide a status update	60
subjective report attribute a subjective report	50
substitute decision-maker verify substitute decision-maker authority	60
supervision document required supervision	41
suspected culprit identify the suspected culprit	23
systems approach use a systems approach	68
teach-back use teach-back	32
team decision record the team decision	42
template text review template text	51
temporal association describe a temporal association	23
therapy recommendation clarify the therapy recommendation	42
time course describe the time course	14
time stamp include the time stamp	6
transfer ability assess transfer ability	41
unable to ascertain document information as unable to ascertain	24
unassessed task identify an unassessed task	42
understanding verified document understanding verified	33
update commitment honor the update commitment	60
urgent review request an urgent review	14
validation provide appropriate validation	59
wheeled walker use a wheeled walker	41
written reinforcement provide written reinforcement	33

Original cases. Clear boundaries.

These are original fictional teaching conversations, not transcripts of real workers. Names, figures, organizations, and incidents are invented. Sources support terminology and communication aims, not the fictional results or an endorsement of this book.

[1] Agency for Healthcare Research and Quality. TeamSTEPPS: Communication.

[Open the original source](#)

Background on structured handoffs, SBAR, and checking back. The original simulations do not prescribe clinical response thresholds. Checked 30 September 2026.

[2] Agency for Healthcare Research and Quality. Use the Teach-Back Method: Tool 5.

[Open the original source](#)

Background on checking the clarity of an explanation through a patient's own words and clarifying misunderstandings. Checked 30 September 2026.

[3] National Institute for Health and Care Excellence. Drug Allergy: Recommendations, section 1.2.

[Open the original source](#)

UK guidance supporting the distinction between reported reactions, documented allergy status, and information requiring clarification. Checked 30 September 2026.

[4] Nursing and Midwifery Council. The Code, sections 10 and 16.

[Open the original source](#)

UK professional standards used as background for accurate records and raising safety concerns, not as a universal legal checklist. Checked 30 September 2026.

Phil Stilwell

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